Meadows Home Health

1641 East Flamingo RoadSte 8
Las Vegas NV 89119
Phone: (702)405-4415 Fax: (702)405-4411
NPI: 1699909077

OASIS

STAFF INFO
Name: Garcia Alfonso, Kelvyn RN
Patient INFO
Name: FLORES-MARTINEZ, DIEGO
DOB: 1987-07-09

OASIS Assessment: OASIS E1 - START OF CARE

ADMINISTRATIVE INFORMATION

(M0010) Agency Medicare Provider Number
297176

(M0014) Branch State
Branch State: **Nevada**

(M0016) Branch ID Number

(M0018) Attending Physician (NPI)
1407841745

Physician:
SILVERBERG, DAVID MR.

Address:
3196 S MARYLAND PARKWAY LAS VEGAS NV 89109

☐ **Not Applicable**

(M0020) Patient ID number
3187

(M0030) Start of Care Date
03/26/2026

(M0032) Resumption of Care Date
mm/dd/yyyy

☑ **Not Applicable**

(M0040) Patient Name

First Name:	Middle Name:	Last Name:	Suffix:
DIEGO		FLORES-MARTINEZ	

(M0050) Patient State of Residence
State of Residence: **Nevada**

(M0060) Patient Zip Code
89121

(M0063) Medicare Number

☑ **Not Applicable**

(M0064) Social Security Number

☑ **Not Applicable**

(M0065) Medicaid Number

☑ **Not Applicable**

(M0066) Birth Date *
07/09/1987

(M0069) Gender *
Gender: **Male**

(M0080) Discipline Completing Assessment:
● 1. RN ○ 2. PT ○ 3. SLP/ST ○ 4. OT

Assigned Staff:
Garcia Alfonso, Kelvyn RN

(M0090) Date Assessment Completed:
03/26/2026

(M0100) Assessment Reason
● 1. Start of Care

(A1005) Ethnicity Are you of Hispanic, Latino/a, or Spanish origin? Check all that apply

- ☐ A. No, not of Hispanic, Latino/a, or Spanish origin
- ☐ B. Yes, Mexican, Mexican American, Chicano/a
- ☐ C. Yes, Puerto Rican
- ☐ D. Yes, Cuban
- ☑ E. Yes, Another Hispanic, Latino, or Spanish origin
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond

(A1010) Race What is your race?

- ☑ A. White
- ☐ B. Black or African American
- ☐ C. American Indian or Alaska Native
- ☐ D. Asian Indian
- ☐ E. Chinese
- ☐ F. Filipino
- ☐ G. Japanese
- ☐ H. Korean
- ☐ I. Vietnamese
- ☐ J. Other Asian
- ☐ K. Native Hawaiian
- ☐ L. Guamanian or Chamorro
- ☐ M. Samoan
- ☐ N. Other Pacific Islander
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond
- ☐ Z. None of the above

(A1110) Language

A. What is your preferred language?
Spanish.

B. Do you need or want an interpreter to communicate with a doctor or health care staff?
○ 0 - No ● 1 - Yes ○ 9 - Unable to determine

(M0102) Date of Physician-ordered Start of Care (Resumption of Care)
If the physician indicated a specific start of care (resumption of care) date when the patient was referred for home health services, record the date specified.
Skip to A1250, Transportation, if date entered:
03/26/2026

☐ **NA - No specific SOC/ROC date ordered by physician**

(M0104) Date of Referral:
Indicate the date that the written or verbal referral for initiation or resumption of care was received by the HHA:
mm/dd/yyyy

(M0150) Current Payment Sources for Home Care: (Mark all that apply.) *

- [ ] 0. None; no charge for current services
- [ ] 1. Medicare (Traditional fee-for-service)
- [ ] 2. Medicare (HMO/managed care/Advantage plan)
- [ ] 3. Medicaid (Traditional fee-for-service)
- [ ] 4. Medicaid (HMO/managed care)
- [ ] 5. Worker's compensation
- [ ] 6. Title programs (for example, Title III, V, or XX)
- [ ] 7. Other government (for example, TriCare, VA)
- [x] 8. Private insurance
- [ ] 9. Private HMO/managed care
- [ ] 10. Self-pay
- [ ] 11. Other (Specify):
- [ ] UK - Unknown

(A1250) Transportation Has lack of transportation kept you from medical appointments, meetings, work, or from getting things needed for daily living?

- [ ] A. Yes, it has kept me from medical appointments or from getting my medications
- [x] B. Yes, it has kept me from non-medical meetings, appointments, work, or from getting things that I need
- [ ] C. No
- [ ] X. Patient unable to respond
- [ ] Y. Patient declines to respond

(M1000) Inpatient Facilities that patient was discharged within the past 14 days? (Mark all that apply.)

- [ ] 1. Long-term nursing facility (NF)
- [ ] 2. Skilled nursing facility (SNF/TCU)
- [ ] 3. Short-stay acute hospital (IPPS)
- [x] 4. Long-term care hospital (LTCH)
- [ ] 5. Inpatient rehabilitation hospital or unit (IRF)
- [ ] 6. Psychiatric hospital or unit
- [ ] 7. Other (specify):
- [ ] NA - Patient was not discharged from an inpatient facility _ **[Go to M1021]**

(M1005) Inpatient Discharge Date (most recent):

mm/dd/yyyy

- [x] UK - Unknown

Primary Reason for Home Health/Medical Necessity:

Patient is a 38-year-old male admitted to Meadows Home Health. The patient is considered homebound because the patient has recent displaced fractures of the head and neck of the right radius; with recent surgery and a non-removable surgical dressing to the right elbow and requires the assistance of another person to safely leave the home. Objective findings supporting homebound status include postoperative right elbow wound not observable due to non-removable dressing, frequent activity-provoked pain of the right elbow/shoulder (pain 3/10 with activity), documented need to maintain limb rest and support during movement, and a documented fall risk (MAHC score 5) while the patient lives alone. Leaving home would require a considerable and taxing effort because transfers, car entry/exit, negotiating steps/curbs or uneven surfaces, and maintaining wound/dressing integrity would provoke pain, risk wound disruption or infection, and exceed the patient's current functional capacity without human assistance; skilled staff documented the patient "needs assistance to leave the place of residence." Allowable absences are limited to brief, medically necessary outings (for example, physician or wound care/therapy appointments); extended or routine community outings are unsafe at this time. These objectively documented functional limitations, postoperative wound care needs, pain and impaired mobility demonstrate that the need for medical necessity for skilled home health services (skilled nursing for wound/pain management and safety monitoring, and skilled therapy for mobility and transfer training) to support safe recovery in the home setting.

Homebound Reason

- [ ] Document as Clinical Narrative
- [ ] Patient is not Homebound

Criteria One (One must be met): *Describe what assistive devices the patient is dependent on (if any) and specifically why the patient condition warrants the special transportation or the assistance of another person or describe the condition that makes leaving home medically contraindicated.*

Criteria 1 A: Because of illness or injury

- [ ] Patient needs the aid of supportive devices
  - [ ] Crutches
  - [ ] Cane
  - [ ] Manual Wheel Chair
  - [ ] Walker
  - [ ] Motorized Wheelchair
  - [ ] Scooter
- [ ] Patient Needs the use of special transportation.
- [x] Patient needs the assistance of another person to leave the place of residence

Criteria 1 B: Has a condition such as leaving his or her home is medically contraindicated.

- [ ] Status post surgery
- [ ] Angina upon minimal exertion
- [ ] Immuno deficiency

Other Homebound Reason :

Criteria Two (Both must be met)

Criteria 2 A: There must be exist a normal inability to leave home.

- [ ] Dyspnea on minimal exertion
- [ ] Bedbound
- [ ] Chair bound
- [ ] Dementia
- [ ] Blindness
- [ ] Angina upon minimal exertion
- [ ] Paralysis
  - [ ] Quadriplegic or
  - [ ] Paraplegic or
  - [ ] Hemiplegia
- [x] Unable to tolerate standing or walking for extended periods
- [x] Weakness and pain from recent surgery
- [x] Experiencing pain impairs/restricts mobility
- [ ] Visually impaired-requires constant attendance/assist
- [ ] Balance impaired with potential for falls; uses walls and furniture to ambulate
- [ ] Cognitively impaired, unable to safely leave home without assist
- [x] Leaving home is exhausting, extended rest periods to recover
- [x] Infectious disease process causing weakness and exhaustion
- [ ] Extended respiratory recovery after exertion
- [ ] Narcotics impaired ability to safely leave home unassisted
- [ ] Requires human assist due to unsteady gait/poor balance
- [ ] Bedbound and requires EMS support

Other Homebound Reason :

Criteria 2 B: Leaving home must require a considerable and taxing effort

- [ ] Special transportation
- [x] Severe pain on ambulation
- [ ] Leaving familiar environment causes fear and confusion
- [ ] Leaving home is strenous requiring extended rest to recover
- [ ] Fatigue to arrange/use appropriate transportation
- [ ] Requires assistive device to ambulate
- [ ] Exhausting to navigate vehicle and require human assist
- [ ] Requires assist to manage equipment and public transportation
- [ ] Needs assistance to handle and transport equipment
- [x] Activities restricted due to pain limitations

Other Homebound Reason :

Prognosis

- ( ) 1-Poor
- ( ) 2-Guarded
- (x) 3-Fair
- ( ) 4-Good
- ( ) 5-Excellent

Functional Limitations

- ( ) 1 - Amputation
- ( ) 2 - Bowel/Bladder (Incontinence)
- ( ) 3 - Contracture
- ( ) B- Other (Specify)
- ( ) 4 - Hearing
- ( ) 5 - Paralysis
- (x) 6 - Endurance
- (x) 7 - Ambulation
- ( ) 8 - Speech
- ( ) 9 - Legally blind
- ( ) A - Dyspnea with minimal exertion

Emergency Preparedness:

Educational materials provided to suggest/assist with emergency management/decision making priorities
Obtained the patient's emergency contact numbers for the medical record
Written materials to restate/reinforce the emergency preparedness procedures given to the Patient and representative
Discussed the development of the patient's individualized emergency preparedness plan of care, including self-care readiness and the procedure to follow up with the HHA in the event services are interrupted
State and local emergency preparedness officials notified about the possible need for evacuation
Discussed patient specific emergency planning options
List of recommended items to have prepared/ready and available in the event of an emergency.

Acuity Level:
Medium Risk or Medium Priority

Emergency Triage:
Category 2 - Not life threatening but would suffer severe adverse effect

Additional Emergency Preparedness Information

- [x] Needs assistance during an emergency
- [x] Contact made with local/state emergency preparedness officials regarding patient in need of help during an evacuation
- [x] Medical Needs/Equipment (i.e. bedbound, oxygen, vent, IV cardiac meds other DME)

Comments:

Advance Directives

Patient has Advance Directives? ( ) Yes (x) No

If you have checked "yes", fill the following fields:

- [ ] Healthcare Representative
- [ ] Do Not Resuscitate
- [ ] Power of Attorney
- [ ] Living Will
- [ ] Conservator Assigned
- [ ] Organ Donor
- [ ] Funeral Arrangements Made
- [ ] Full Code
- [ ] The patient has not executed any advance directives to health care.
- [x] The patient was provided with written and verbal information on advance directives.

Comments:

Has surrogate? ( ) Yes (x) No

Surrogate Name:

Surrogate Phone:

Cultural

Primary Language?
- [ ] English
- [x] Spanish
- [ ] Chinese
- [ ] Vietnamese

Other:

Is religion important to the patient? ( ) Yes (x) No

Patient's religious preference:

Patient's primary source of emotional support:

Past Medical History (Mark all that apply)

- [ ] CHF
- [ ] Chest Pain
- [ ] HTN
- [ ] Cardiomyopathy
- [ ] MI
- [ ] PVD
- [ ] Arrhythmia
- [ ] CAD
- [ ] Murmur
- [ ] Cancer: (Specify Type): In remission? ( ) Yes ( ) No
- [ ] Osteoathritis/DJD: (Specify site affected):
- [ ] Joint Replacement: (Specify joint):

- [ ] Rheumatoid Arthritis
- [x] Gait Problems
- [x] Fractures
- [ ] Falls
- [ ] CVA
- [ ] TIA
- [ ] MS
- [ ] Hemiplegia
- [ ] Seizures
- [ ] Headaches
- [ ] Dizziness/Vertigo

- [ ] IBS
- [ ] Crohn's Disease
- [ ] Diverticulitis/Diverticulosis
- [ ] Constipation
- [ ] Diarrhea
- [ ] Fecal Incontinence
- [ ] Liver/Gallbladder Problems: (Specify):

- [ ] Depression
- [ ] Anxiety
- [ ] Dementia
- [ ] Substance Abuse: (Specify):
- [ ] Alzheimer's
- [ ] Mental Disorder: (Specify):

- [ ] Pressure Ulcer
- [ ] Stasis Ulcer
- [ ] Diabetic Ulcer
- [ ] Trauma Wound
- [ ] Other: (Specify):
- [ ] Chronic Kidney Disease
- [ ] Renal Failure
- [ ] Dialysis

- [ ] Anemia
- [ ] Abnormal Coagulation
- [ ] Blood Clots
- [ ] Diabetes
- [ ] Thyroid Problems

- [ ] COPD
- [ ] Asthma
- [ ] Emphysema
- [ ] Chronic Obstructive Bronchitis
- [ ] Chronic Obstructive Asthma

- [ ] Urinary Incontinence
- [ ] Urinary Retention
- [ ] BPH
- [ ] Recent/Frequent UTI
- [ ] Tuberculosis
- [ ] Hepatitis: (Specify):
- [ ] Infectious Disease: (Specify):

- [x] Tobacco Dependence

Type: Nicotine Dependence
Amount:
Length of Time Used:

Past Surgical History:

Health Screening

Last Cholesterol Level: mm/dd/yyyy
Last Mammogram: mm/dd/yyyy

Does patient perform monthly self breast exam? ◯ Yes ◯ No

Last Pap Smear: mm/dd/yyyy
Last PSA: mm/dd/yyyy

Last Prostate Exam: mm/dd/yyyy
Last Colonoscopy: mm/dd/yyyy

(M1100) Patient Living Situation. Which of the following best describes the patient's residential circumstance and availability of assistance?

Living Arrangement	Around the clock	Regular daytime	Regular Night time	Short-term Assistance	No Assistance available
A. Patient lives alone	◯ Patient Lives alone around the clock	◯ Patient Lives alone Regular daytime	◯ Patient Lives alone Regular Night time	◯ Patient Lives alone Short-term Assistance	◯ Patient Lives alone No Assistance available
B. Patient lives with other person(s) in the home	● Patient lives with other person(s) in the home Around the clock	◯ Patient lives with other person(s) in the home Regular daytime	◯ Patient lives with other person(s) in the home Regular Night time	◯ Patient lives with other person(s) in the home Short-term Assistance	◯ Patient lives with other person(s) in the home No Assistance available
C. Patient lives in congregate situation (for example, assisted living, residential care home)	◯ Patient lives in congregate situation (for example, assisted living, residential care home) around the clock	◯ Regular daytime	◯ Patient lives in congregate situation Regular Night time	◯ Patient lives in congregate situation Short-term Assistance	◯ Patient lives in congregate situation No Assistance available

Caregiver Contact Info:

Primary Caregiver:
Relationship to Patient
Contact Number

Comments:

Name of Facility:

Able to safely care for patient? ● Yes ◯ No

Phone:

ACTIVE DIAGNOSES

(M1021/1023) Diagnoses and Symptom Control

List each diagnosis for which the patient is receiving home care in Column 1, and enter its ICD-10-CM code at the level of highest specificity in Column 2 (diagnosis codes only – no surgical or procedure codes allowed). Diagnoses are listed in the order that best reflects the seriousness of each condition and supports the disciplines and services provided. Rate the degree of symptom control for each condition in Column 2. ICD-10-CM sequencing requirements must be followed if multiple coding is indicated for any diagnoses.

Code each row according to the following directions for each column:

- **Column** 1: Enter the description of the diagnosis. Sequencing of diagnoses should reflect the seriousness of each condition and support the disciplines and services provided. Column 2: Enter the ICD-10-CM code for the condition described in Column 1 – no surgical or procedure codes allowed. Codes must be entered at the level of highest specificity and ICD-10-CM coding rules and sequencing requirements must be followed. Note that external cause codes (ICD-10-CM codes beginning with V, W, X, or Y) may not be reported in M1021 (Primary Diagnosis) but may be reported in M1023 (Secondary Diagnoses). Also note that when a Z-code is reported in

- **Column 2 :** the code for the underlying condition can often be entered in Column 2, as long as it is an active on-going condition impacting home health care. Rate the degree of symptom control for the condition listed in Column 1. Do not assign a symptom control rating if the diagnosis code is a V, W, X, Y or Z-code. Choose one value that represents the degree of symptom control appropriate for each diagnosis using the following scale: 0 - Asymptomatic, no treatment needed at this time 1 - Symptoms well controlled with current therapy 2 - Symptoms controlled with difficulty, affecting daily functioning; patient needs ongoing monitoring 3 - Symptoms poorly controlled; patient needs frequent adjustment in treatment and dose monitoring 4 - Symptoms poorly controlled; history of re-hospitalizations Note that the rating for symptom control in Column 2 should not be used to determine the sequencing of the diagnoses listed in Column 1. These are separate items and sequencing may not coincide.

PRIMARY DIAGNOSIS
S52.121D - Disp fx of head of r radius, subs for clos fx w routn heal

Onset Date 03/26/2026

Severity
0 | 1 | 2 | (3) | 4 | ^

Onset | (Exacerbate)

OTHER DIAGNOSIS - 1
S52.131D - Disp fx of neck of r radius, subs for clos fx w routn heal

Onset Date 03/26/2026

Severity
0 | 1 | 2 | (3) | 4 | ^

Onset | (Exacerbate)

OTHER DIAGNOSIS - 2
F17.210 - Nicotine dependence, cigarettes, uncomplicated

Onset Date 03/26/2026

Severity
0 | 1 | (2) | 3 | 4 | ^

(Onset) | Exacerbate

(M1028) Active Diagnoses – Comorbidities and Co-existing Conditions – Check all that apply See OASIS Guidance Manual for a complete list of relevant ICD-10 codes.

- [ ] 1. Peripheral Vascular Disease (PVD) or Peripheral Arterial Disease (PAD)
- [ ] 2. Diabetes Mellitus (DM)
- [x] 3. None of the above

VITAL SIGNS & PAIN ASSESSMENT

(M1060) Height and Weight - While measuring, if the number is X.1-X.4 round down; X.5 or greater round up

A. Height (in inches). Record most recent height measure since the most recent SOC/ROC:
65

B. Weight (in pounds). Base weight on most recent measure in last 30 days; measure weight consistently, according to standard agency practice (for example, in a.m. after voiding, before meal, with shoes off, etc.:
178

BMI:
29.62

Comments:

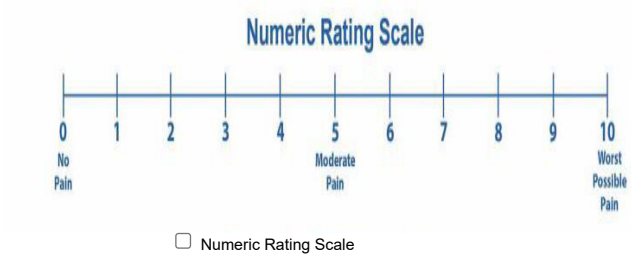
Vital Signs

Temperature: 98.4 Temporal
Respiratory (per minute): 12
Blood Pressure mm/Hg (R): 110 / 72 sitting
Blood Sugar (mg/dl): Non-Fasting
O2 Sat: 97
Comments:

Pulse (per minute): Apical = Radial = 72
Weight: 178 reported
Blood Pressure mm/Hg (L): /

O2 Sat in: Room Air

Pain Assessment



3

Collected Using: [ ] Face Scale [ ] Numeric Rating Scale

Pain Location and Description:

1 **Location**
Right elbow, right shoulder.

Start Date
mm/dd/yyyy

Activity
With Activity×

What makes pain worse?
Activity×

Intensity
3×

Duration

What makes pain better?
Rest×

Does pain affect quality of life?
Yes.

Unable to Assess [ ]

Unable to Assess [ ]

Description
Dull Ache×

Frequency
Frequent×

Type of Intervention

How well it is working

Status

Comments:

(J0510) Pain Effect on Sleep

Ask patient: "Over the past 5 days, how much of the time has pain made it hard for you to sleep at night"

- ○ **0. Does not apply – I have not had any pain or hurting in the past 5 days**
- ● **1. Rarely or not at all**
- ○ **2. Occasionally**
- ○ **3. Frequently**
- ○ **4. Almost constantly**
- ○ **8. Unable to answer**

(J0520) Pain Interference with Therapy Activities

Ask patient: "Over the past 5 days, how often have you limited your participation in rehabilitation therapy sessions due to pain?"

- ● **0. Does not apply – I have not received rehabilitation therapy in the past 5 days**
- ○ **1. Rarely or not at all**
- ○ **2. Occasionally**
- ○ **3. Frequently**
- ○ **4. Almost constantly**
- ○ **8. Unable to answer**

(J0530) Pain Interference with Day-to-Day Activities

Ask patient: "Over the past 5 days, how often you have limited your day-to-day activities (excluding rehabilitation therapy session) because of pain?"

- ○ **1. Rarely or not at all**
- ● **2. Occasionally**
- ○ **3. Frequently**
- ○ **4. Almost Constantly**
- ○ **8. Unable to answer**

MEDICATION & ALLERGIES (INJECTABLE MEDICATIONS)

(N0415) High-Risk Drug Classes: Use and Indication:

	1. Is Taking 1. Is taking Check if the patient is taking any medications by pharmacological classification, not how it is used, in the following classes	2. Indication Noted 2. Indication noted If Column 1 is checked, check if there is an indication noted for all medications in the drug class
A. Antipsychotic	☐	☐
E. Anticoagulant	☐	☐
F. Antibiotic	☐	☐
H. Opioid	☐	☐
I. Antiplatelet	☐	☐
J. Hypoglycemic (including insulin)	☐	☐
Z. None of the Above	☑	

Medication

☐	Start Date	Medication	Strength / Dosage / Frequency	Route	Classification/Indication	Status

Medication Remarks
Medication remarks...

☐ Discontinued Medications

Start Date	Medication	Strength / Dose / Frequency / Route	Classification/Indication	Laboratory	Status	Disc Date	Remarks

IV

Does patient have IV access? ○ **Yes** ● **No**

(M2001) Drug Regimen Review

Did a complete drug regimen review identify potential clinically significant medication issues?

- ● **0. No – No issues found during review _ [Go to M2010, Patient/Caregiver High-Risk Drug Education]**
- ○ **1. Yes – Issues found during review**
- ○ **Not assessed/no information**
- ○ **9. NA – Patient is not taking any medications _ [Go to O0110, Special Treatments, Procedures, and Programs]**

(M2003) Medication Follow-up

Did the agency contact a physician (or physician-designee) by midnight of the next calendar day and complete prescribed/recommended actions in response to the identified potential clinically significant medication issues?

- ○ **0. No**
- ○ **1. Yes**
- ○ **Not assessed/no information**

(M2010) Patient/Caregiver High-Risk Drug Education

Has the patient/caregiver received instruction on special precautions for all high-risk medications (such as hypoglycemics, anticoagulants, etc.) and how and when to report problems that may occur?

- ○ **0. No**
- ○ **1. Yes**
- ● **NA - Patient not taking any high-risk drugs OR patient/caregiver fully knowledgeable about special precautions associated with all high-risk medications**

(M2020) Management of Oral Medications

Patient's current ability to prepare and take all oral medications reliably and safely, including administration of the correct dosage at the appropriate times/intervals. Excludes injectable and IV medications. (NOTE: This refers to ability, not compliance or willingness.)

- ○ **0. Able to independently take the correct oral medication(s) and proper dosage(s) at the correct times.**
- ○ **1. Able to take medication(s) at the correct times if: (a) individual dosages are prepared in advance by another person; OR (b) another person develops a drug diary or chart.**
- ○ **2. Able to take medication(s) at the correct times if given reminders by another person at the appropriate times.**
- ○ **3. Unable to take medication unless administered by another person**
- ● **NA - No oral medications prescribed**

(M2030) Management of Injectable Medications

Patient's current ability to prepare and take all prescribed injectable medications reliably and safely, including administration of correct dosage at the appropriate times/intervals. Excludes IV medications.

- ◯ **0. Able to independently take the correct medication(s) and proper dosage(s) at the correct times**
- ◯ **1. Able to take injectable medication(s) at the correct times if: a. individual syringes are prepared in advance by another person; OR b. another person develops a drug diary or chart.**
- ◯ **2. Able to take medication(s) at the correct times if given reminders by another person based on the frequency of the injection**
- ◯ **3. Unable to take injectable medication unless administered by another person**
- ● **NA - No injectable medications prescribed.**

(O0110) Special Treatments, Procedures, and Programs

Check all of the following treatments, procedures, and programs that apply at discharge.

	On Admission (Check All That Apply)
Cancer Treatment	
A1. Chemotherapy	☐
A2. IV	☐
A3. Oral	☐
A10. Other	☐
B1. Radiation	☐
Respiratory Therapies	
C1. Oxygen Therapy	☐
C2. Continuous	☐
C3. Intermittent	☐
C4. High-Concentration	☐
D1. Suctioning	☐
D2. Scheduled	☐
D3. As needed	☐
E1. Tracheostomy Care	☐
F1. Invasive Mechanical Ventilator	☐
G1. Non-invasive Mech. Vent.	☐
G2. BiPAP	☐
G3. CPAP	☐
Others	
H1. IV Medications	☐
H2. Vasoactive medications	☐
H3. Antibiotics	☐
H4. Anticoagulation	☐
H10. Other	☐
I1. Transfusions	☐
J1. Dialysis	☐
J2. Hemodialysis	☐
J3. Peritoneal dialysis	☐
O1. IV Access	☐
O2. Peripheral	☐
O3. Mid-line	☐
O4. Central (e.g., PICC, tunneled, port)	☐
Z1. None of the Above	☑

Most Recent Immunizations

Pneumonia	◯ Yes	◯ No	● Unknown	Date: mm/dd/yyyy
Flu	◯ Yes	◯ No	● Unknown	Date: mm/dd/yyyy
Tetanus	◯ Yes	◯ No	● Unknown	Date: mm/dd/yyyy
TB	◯ Yes	◯ No	● Unknown	Date: mm/dd/yyyy
TB Exposure	◯ Yes	◯ No	● Unknown	Date:

mm/dd/yyyy

Hepatitis B
◯ Yes ◯ No ● Unknown Date: mm/dd/yyyy

Additional Immunizations

Immunization Name:
◯ Yes ◯ No ● Unknown Date: mm/dd/yyyy

Immunization Name:
◯ Yes ◯ No ● Unknown Date: mm/dd/yyyy

Comments:

Vaccination Record

1st Dose COVID-19
Product Name/Manufacturer (Lot No.): Date: mm/dd/yyyy Healthcare Professional or Clinic Site:

2nd Dose COVID-19
Product Name/Manufacturer (Lot No.): Date: mm/dd/yyyy Healthcare Professional or Clinic Site:

Booster 1
Product Name/Manufacturer (Lot No.): Date: mm/dd/yyyy Healthcare Professional or Clinic Site:

Booster 2
Product Name/Manufacturer (Lot No.): Date: mm/dd/yyyy Healthcare Professional or Clinic Site:

Allergies

Name	Reaction	Start Date
None known		

Add Allergies

NEUROLOGICAL (HEAD, MOOD, EYES, EARS)

Neuro/Emotional/Behavioral Assessment

Conditions of Participation
Neuromascular

Impairment:
☑ No Problems identified
☐ Speech ☐ Hearing ☐ Visual ☐ Tremors ☐ Numbness ☐ Tingling ☐ Vertigo ☐ Ataxia ☐ Decreased Sensitivity

Others (Specify):

Psychosocial

☑ WNL (Within Normal Limits)
☐ Poor Home Environment
☐ Impaired Decision Making
☐ Poor Coping Skills
☐ Demonstrated/Expressed Anxiety
☐ Agitated
☐ Inappropriate Behavior
☐ Depressed Mood
☐ Irritability

☐ Document as Clinical Narrative

Educational level
High school.

Primary Language

Unable to: ☐ *Read* ☐ *Write*

Barriers:

Language
● Needs interpreter

Learning
◯ Mental ● Physical ◯ Psychosocial ● Functional

Emotions Reported:

◯ Angry
◯ Discouraged
◯ Withdrawn

◯ Flat affect
◯ Difficulty coping
◯ Disorganized

◯ Depressed ◯ Recent ◯ Long Term

Treatment:

Evidence of abuse/neglect/exploitation:

- ◯ Potential
- ◯ Verbal/Emotional
- ◯ Physical
- ◯ Actual
- ◯ Financial
- ● No Evidence of Abuse/Neglect/Exploitation

Unable to cope with altered health status, patient:

- ◯ Lacks motivation
- ◯ Has unrealistic expectations
- ● Denies problems
- ◯ Unable to recognize problems

Sleep/Rest

- ● Adequate
- ◯ Inadequate

No. of hours slept per night: 7

Comments

Spiritual / Cultural implications that impact care

Spiritual resource

☐ Inappropriate responses to caregivers/clinician

☐ Inappropriate follow-through in past

Did patient drive a vehicle before admission? ☑ Yes ☐ No ☐ NA

Did the patient have a job before admission? ◯ Yes ◉ No

If yes, do they want to drive post-discharge? ☑ Yes ☐ No

If yes, do they want to return to work post-discharge? ◯ Yes ◯ No

(B1300) Health Literacy

How often do you need to have someone help you when you read instructions, pamphlets, or other written material from your doctor or pharmacy?

- ◯ 0. Never
- ● 1. Rarely
- ◯ 2. Sometimes
- ◯ 3. Often
- ◯ 4. Always
- ◯ 7. Patient declines to respond
- ◯ 8. Patient unable to respond

(C0100) Should Brief Interview for Mental Status (C0200-C0500) be Conducted?

Attempt to conduct interview with all patients. ◯ **No (patient is rarely/never understood)** ● **Yes** ◯ **Not assessed/no information**

Brief Interview for Mental Status (BIMS)

(C0200) Repetition of Three Words

Ask patient: **"I am going to say three words for you to remember. Please repeat the words after I have said all three. The words are: SOCK , BLUE , and BED . Now tell me the three words."**

Number of words repeated after first attempt ◯ **None** ◯ **One** ◯ **Two** ● **Three** ◯ **Not Assessed**

After the patient's first attempt, repeat the words using cues (**"sock, something to wear; blue, a color; bed, a piece of furniture"**). You may repeat the words up to two more times.

(C0300) Temporal Orientation (Orientation to year, month, and day)

Ask patient: **"Please tell me what year it is right now?"**

A) Able to report correct year

- ◯ 0. Missed by > 5 years or no answer
- ◯ 1. Missed by 2-5 years
- ◯ 2. Missed by 1 year
- ● 3. Correct
- ◯ Not assessed/no information

Ask patient: **"What month are we in right now?"**

B) Able to report correct month

- ◯ 0. Missed by > 1 month or no answer
- ◯ 1. Missed by 6 days to 1 month
- ● 2. Accurate within 5 days
- ◯ Not assessed/no information

Ask patient: **"What day of the week is today?"**

C) Able to report correct day of the week

- ◯ 0. Incorrect or no answer
- ● 1. Correct
- ◯ Not assessed/no information

(C0400). Recall

Ask patient: **"Let's go back to an earlier question. What were those three words that I asked you to repeat?" If unable to remember a word, give cue (something to wear; a color; a piece of furniture) for that word."**

A) Able to recall "sock"

- ◯ 0. No – could not recall
- ◯ 1. Yes, after cueing ("something to wear")
- ● 2. Yes, no cue required
- ◯ Not assessed/no information

B) Able to recall "blue"

- ◯ 0. No – could not recall
- ◯ 1. Yes, after cueing ("a color")
- ● 2. Yes, no cue required
- ◯ Not assessed/no information

C) Able to recall "bed"

- ◯ 0. No – could not recall
- ◯ 1. Yes, after cueing ("a piece of furniture")
- ● 2. Yes, no cue required
- ◯ Not assessed/no information

C0500. BIMS Summary Score

Add scores for questions C0200-C0400 and fill in total score (00-15) **Enter 99 if the patient was unable to complete the interview** 15

Mental Status

- ● 1 - Oriented
- ◯ 2 - Comatose
- ◯ 3 - Forgetful
- ◯ 4 - Depressed
- ◯ 5 - Disoriented
- ◯ 6 - Lethargic
- ◯ 7 - Agitated
- ● 8 - Alert
- ◯ 9 - Confused
- ◯ 10 - Dementia
- ◯ 11 - Assaultive
- ◯ 12 - Delirious
- ◯ 13 - Wanders
- 14 - Other

Describe the patient's mental status. Include general appearance, behaviors, emotional responses, mental functioning, overall social interaction as well as both clinical objective observations and subjective descriptions reported during this visit.

Patient is alert, oriented, able to follow instructions and answer to questions accordingly, with episodes of depression. Patient cooperative with exam and exhibits pleasant and calm behavior. Dress is appropriate, well-groomed, and proper hygiene. Patients affect and facial expressions are appropriate to the situation, at the time of visit Patient did not express suicidal/homicidal ideation.

Has there been a sudden/acute change in their mental status? ☐ Yes ☑ No Reported by: ☐ If yes, did the change coincide with something else like medication change, fall, or loss of a loved one? ☑ No ☐ Yes

Comments:

(C1310) Signs and Symptoms of Delirium (from CAM©)

A. Acute Onset of Mental Status Change

Is there evidence of an acute change in mental status from the patient's baseline?

● **0. No** ◯ **1. Yes**

B. Inattention

Did the patient have difficulty focusing attention, for example, being easily distractible or having difficulty keeping track of what was being said?

- ● **0. Behavior not present**
- ◯ **2. Behavior present, fluctuates (comes and goes, changes in severity)**

- ◯ 1. Behavior continuously present, does not fluctuate
- ◯ Not assessed/no information

C. Disorganized thinking

Was the patient's thinking disorganized or incoherent (rambling or irrelevant conversation, unclear or illogical flow of ideas, or unpredictable switching from subject to subject)?

- ☑ 0. Behavior not present
- ◯ 1. Behavior continuously present, does not fluctuate
- ◯ 2. Behavior present, fluctuates (comes and goes, changes in severity)
- ◯ Not assessed/no information

D. Altered level of consciousness

Did the patient have altered level of consciousness, as indicated by any of the following criteria?

- **vigilant** – startled easily to any sound or touch lethargic – repeatedly dozed off when being asked questions, but responded to voice or touch
- **stuporous** – very difficult to arouse and keep aroused for the interview
- **comatose** – could not be arouse

- ☑ 0. Behavior not present
- ◯ 1. Behavior continuously present, does not fluctuate
- ◯ 2. Behavior present, fluctuates (comes and goes, changes in severity)
- ◯ Not assessed/no information

(D0150) Patient Mood Interview (PHQ-2 to 9)

Say to patient: **"Over the last 2 weeks, have you been bothered by any of the following problems?"**
If symptom is present, **enter 1 (yes) in column 1 , Symptom Presence** . If yes in column 1, then ask the patient: **"About how often have you been bothered by this?"**
Read and show the patient a card with the symptom frequency choices. Indicate response in column 2, Symptom Frequency

1) Symptom Presence
- **0. No (enter 0 in column 2)**
- **1. Yes (enter 0-3 in column 2)**
- **9. No response (leave column 2 blank)**

2) Symptom Frequency
- 0 - **Never or 1 day**
- 1 - **2-6 days (several days)**
- 2 - **7-11 days (half or more of the days)**
- 3 - **12-14 days (nearly every day)**

	Presence	Frequency
A. Little interest or pleasure in doing things	No ×	Never or 1 day
B. Feeling down, depressed, or hopeless	No ×	Never or 1 day
If either **D0150A2 or D0150B2** is coded **2 or 3** , **CONTINUE** asking the questions below. If not, **END the PHQ interview**		
C. Trouble falling or staying asleep, or sleeping too much	^	^
D. Feeling tired or having little energy	^	^
E. Poor appetite or overeating	^	^
F. Feeling bad about yourself – or that you are a failure or have let yourself or your family down	^	^
G. Trouble concentrating on things, such as reading the newspaper or watching television	^	^
H. Moving or speaking so slowly that other people could have noticed. Or the opposite – being so fidgety or restless that you have been moving around a lot more than usual	^	^
I. Thoughts that you would be better off dead, or of hurting yourself in some way	^	^
D0160. Total Severity Score Add scores for all frequency responses in Column 2, Symptom Frequency. Total score must be between 02 and 27. Enter 99 if unable to complete interview (i.e., Symptom Frequency is blank for 3 or more required items)		0

(D0700) Social Isolation

How often do you feel lonely or isolated from those around you?

- ☑ 0. Never
- ◯ 1. Rarely
- ◯ 2. Sometimes
- ◯ 3. Often
- ◯ 4. Always
- ◯ 7. Patient declines to respond
- ◯ 8. Patient unable to respond

(M1740) Cognitive, behavioral, and psychiatric symptoms that are demonstrated at least once a week (Reported or Observed): (Mark all that apply.

- ☐ 1. Memory deficit: failure to recognize familiar persons/places, inability to recall events of past 24 hours, significant memory loss so that supervision is required
- ☐ 2. Impaired decision-making: failure to perform usual ADLs or IADLs, inability to appropriately stop activities, jeopardizes safety through actions
- ☐ 3. Verbal disruption: yelling, threatening, excessive profanity, sexual references, etc.
- ☐ 4. Physical aggression: aggressive or combative to self and others (for example, hits self, throws objects, punches, dangerous maneuvers with wheelchair or other objects)
- ☐ 5. Disruptive, infantile, or socially inappropriate behavior (excludes verbal actions)
- ☐ 6. Delusional, hallucinatory, or paranoid behavior
- ☑ 7. None of the above behaviors demonstrated

(M1745) Frequency of Disruptive Behavior Symptoms (Reported or Observed):

- ☑ 0. Never
- ◯ 1. Less than once a month
- ◯ 2. Once a month
- ◯ 3. Several times each month
- ◯ 4. Several times a week
- ◯ 5. At least daily

(M1700) Cognitive Functioning

Patient's current (day of assessment) level of alertness, orientation, comprehension, concentration, and immediate memory for simple commands

- ☑ 0. Alert/oriented, able to focus and shift attention, comprehends and recalls task directions independently.
- ◯ 1. Requires prompting (cuing, repetition, reminders) only under stressful or unfamiliar conditions.
- ◯ 2. Requires assistance and some direction in specific situations (for example, on all tasks involving shifting of attention) or consistently requires low stimulus environment due to distractibility
- ◯ 3. Requires considerable assistance in routine situations. Is not alert and oriented or is unable to shift attention and recall directions more than half the time.
- ◯ 4. Totally dependent due to disturbances such as constant disorientation, coma, persistent vegetative state, or delirium.

(M1710) When Confused (Reported or Observed Within the Last 14 Days):

- ☑ 0. Never
- ◯ 1. In new or complex situations only
- ◯ 2. On awakening or at night only
- ◯ 3. During the day and evening, but not constantly
- ◯ 4. Constantly
- ◯ 5. Patient nonresponsive

(M1720) When Anxious (Reported or Observed Within the Last 14 Days):

- ☑ 0. None of the time
- ◯ 1. Less often than daily
- ◯ 2. Daily, but not constantly
- ◯ 3. All of the time
- ◯ NA - Patient nonresponsive

Eyes and Ears

(B1000) Vision

Ability to see in adequate light (with glasses or other visual appliances)

- ○ 0. Adequate – sees fine detail, such as regular print in newspapers/books
- ☑ 1. Impaired – sees large print, but not regular print in newspapers/books
- ○ 2. Moderate impaired – limited vision; not able to see newspaper headlines but can identify objects
- ○ 3. Highly impaired – object identification in question, but eyes appear to follow objects
- ○ 4. Severely impaired – no vision or sees only light, colors or shapes; eyes do not appear to follow objects

Vision cont.

☐ No Problems Identified

☑ Glasses ☐ Macular Degeneration ☐ Blurred Vision ☐ Glaucoma

☐ Contact Lenses: ☐ Right ☐ Left ☐ Cataracts

☐ Blindness(Specify): ☐ Others (Specify):

Comments:

Pupils

☐ No Problems Identified

☑ PERRLA ☐ Anisocoria ☐ Right ☐ Left

☐ Others(Specify):

Comments:

(B0200) Hearing

Ability to hear (with hearing aid or hearing appliances if normally used)

- ☑ 0. Adequate – no difficulty in normal conversation, social interaction, listening to TV
- ○ 1. Minimal difficulty – difficulty in some environments (e.g., when person speaks softly, or setting is noisy)
- ○ 2. Moderate difficulty – speaker has to increase volume and speak distinctly
- ○ 3. Highly impaired – absence of useful hearing

Hearing cont.

☑ No Problems Identified

☐ Right Hearing Impairment ☐ Left Hearing Impairment ☐ Deaf

☐ Speech Impairment (Specify):

Comments:

CARDIOPULMONARY (CHEST & THORAX)

Heart Sounds

☑ Normal ☑ Regular ☐ Irregular ☐ Murmur

☐ Gallops ☐ Clicks ☐ Friction Rub

Comments (If Abnormal (Explain)):

Edema

Upper: ☐ Right Extremities ☐ Left Extremities

Lower: ☐ Right Extremities ☐ Left Extremities

Others:

Comments:

. ☐ Cramps ☐ Claudication ☐ Fatigue

Capillary Refill: ☐ > 3 secs ☑ < 3 secs

PT/INR: _____ / _____

Peripheral Pulses

Radial:	☑ Strong/Bounding	☐ Weak	☐ Thready	☐ Absent
Pedal:	☑ Strong/Bounding	☐ Weak	☐ Thready	☐ Absent

Comments:

Chest/Thorax

Chest Pain

☑ Denies ☐ Anginal ☐ Postural ☐ Localized ☐ Substernal ☐ Sharp

☐ Dull ☐ Aching ☐ Radiating ☐ Viselike ☐ Stabbing

☐ Others (Specify):

Associated with: ☐ Shortness of Breath ☐ Activity ☐ Rest ☐ Frequency/Duration:

Comments:

(M1400) When is the patient dyspneic or noticeably Short of Breath?

- ☑ 0. Patient is not short of breath
- ○ 1. When walking more than 20 feet, climbing stairs
- ○ 2. With moderate exertion (for example, while dressing, using commode or bedpan, walking distances less than 20 feet)
- ○ 3. With minimal exertion (for example, while eating, talking, or performing other ADLs) or with agitation
- ○ 4. At rest (during day or night)

Respiratory Status

☐ Dyspnea: ☐ At Rest ☐ With Exertion/Activity ☐ Accessory Muscle Used

☐ Orthopnea ☐ Retraction ☐ Stridor

O2 _____ lpm O2 Saturation _____ % ☐ PRN ☐ Continuous

☐ Mechanical Ventilator: ☐ Intubated ☐ Tracheostomy Others (Specify):

Comments:

Lung Sounds

Anterior: Right: **Clear** Left: **Clear**
Posterior: Upper Right: **Clear** Upper Left: **Clear** Lower Right: **Clear** Lower Left: **Clear**
Comments:

Cough

- [x] None
- [ ] Dry
- [ ] Acute
- [ ] Chronic
- [ ] Non-Productive
- [ ] Productive: [ ] Not Assessed/No Information [ ] Yes [ ] No
- Suction: [ ] Not Assessed/No Information [ ] Yes [ ] No
- [ ] Hemoptysis Frequency: Amount:

Comments:

GASTROINTESTINAL & GENITOURINARY ASSESSMENT

Gastrointestinal (Mouth - Anus)

Mouth and Throat

- [x] No Problems Identified
- [ ] Dentures: [ ] Upper [ ] Lower [ ] Partial
- [ ] Bleeding Gums/Gingivitis [ ] Toothache [ ] Ulcerations [ ] Lesion
- [ ] Dysphagia [ ] Hoarseness [ ] Sore Throat [ ] Masses/Tumors

Comments:

Appetite

- [x] Good
- [ ] Fair
- [ ] Poor
- Intake: [ ] Adequate [ ] Inadequate

Abnormalities:

- [ ] Anoxeria
- [ ] Cachexia
- [ ] Nausea
- [ ] Vomiting
- [ ] Decreased Appetite
- [ ] Reflux/Indigestion
- [ ] Diarrhea
- [ ] Constipation
- [ ] Bowel Incontinence
- [ ] Other (Specify):

Comments:

(M1870) Feeding or Eating

Current ability to feed self meals and snacks safely. Note: This refers only to the process of ***eating***, ***chewing***, and ***swallowing***, not preparing the food to be eaten.

- (●) **0. Able to independently feed self.**
- ( ) **1. Able to feed self independently but requires: (a) meal set-up; OR (b) intermittent assistance or supervision from another person; OR (c) a liquid, pureed or ground meat diet.**
- ( ) **2. Unable to feed self and must be assisted or supervised throughout the meal/snack.**
- ( ) **3. Able to take in nutrients orally and receives supplemental nutrients though a nasogastric tube or gastrostomy.**
- ( ) **4. Unable to take in nutrients orally and is fed nutrients through a nasogastric tube or gastrostomy.**
- ( ) **5. Unable to take in nutrients orally or by tube feeding.**

Feeding Cont.

- [ ] Formula
- [ ] Continuous Feeding cc: hours:
- [ ] Bolus:
- [ ] Gravity
- [ ] Pump cc: every hour(s):

Flushing Protocol:

PEG/G-Tube Placement:

- [ ] Placement Checked
- [ ] Residual Checked Amount:

Comments:

(K0520) Nutritional Approaches

	On Admission (Check All That Apply)
A. Parenteral/IV feeding	[ ]
B. Feeding tube (e.g., nasogastric or abdominal (PEG))	[ ]
C. Mechanically altered diet – require change in texture of food or liquids (e.g., pureed food, thickened liquids)	[ ]
D. Therapeutic diet (e.g., low salt, diabetic, low cholesterol)	[ ]
Z. None of the above	[x]

#16 Nutritional Requirements

- [ ] NPO
- [ ] No Added Salt
- [ ] Low Salt
- [ ] Low Fat
- [ ] Low Cholesterol
- [ ] No Concentrated Sweets
- [ ] Controlled Carbohydrates
- [x] Regular
- [ ] Fluid Restriction:

Other:

Nutritional Health Screen	Score	Nutritional Health Screen	Score
( ) Without reason, has lost more than 10 lbs in the last 3 months	15	( ) Does not always have enough money to buy foods need	10
( ) Has an illness or condition that made pt change the type and/or amount of food eaten	10	( ) Eats alone most of the time	5
( ) Has open decubitus, ulcer, burn or wound	10		
( ) Eats fewer than 2 meals a day	10	( ) Takes 3 or more prescribed or OTC medications a day	5
( ) Has a tooth/mouth problem that makes it hard to eat	10	( ) Is not always physically able to cook and/or feed self and has no caregiver to assist	5
( ) Has 3 or more drinks of beer, liquor or wine almost every day	10	( ) Frequently has a diarrhea or constipation	5
		Total	0

- [x] Good Nutritional Status (Score 0 - 25)
- [ ] Moderate Nutritional Status (Score 25 - 54)
- [ ] High Nutritional Status (Score 55 - 100)

Nutritional Status Commments:

Weight

- [x] No Change
- [ ] Weight Gain
- [ ] Weight Loss

Abdomen

- [x] Abdominal Soft/Tender
- [ ] Abdominal Distention
- [ ] Girth(cm):

Comments:

(M1620) Bowel Incontinence Frequency

- [x] 0. Very rarely or never has bowel incontinence
- [ ] 1. Less than once weekly
- [ ] 2. One to three times weekly
- [ ] 3. Four to six times weekly
- [ ] 4. On a daily basis
- [ ] 5. More often than once daily
- [ ] NA - Patient has ostomy for bowel elimination
- [ ] UK - Unknown

(M1630) Ostomy for Bowel Elimination

Does this patient have an ostomy for bowel elimination that (within the last 14 days): a) was related to an inpatient facility stay; or b) necessitated a change in medical or treatment regimen?

- [x] 0. Patient does not have an ostomy for bowel elimination
- [ ] 1. Patient's ostomy was not related to an inpatient stay and did not necessitate change in medical or treatment regimen.
- [ ] 2. The ostomy was related to an inpatient stay or did necessitate change in medical or treatment regimen.

Ostomy/Stoma Appearance

- [ ] Pinkished-Red
- [ ] Moist
- [ ] Gray
- [ ] Bad Odor

Abnormalities:

- [ ] Purple
- [ ] Black in Color
- [ ] Dry
- [ ] Itches/Hurt

Other (Specify):

Bowel Sounds:

Abdominal Quadrants:

Right Lower	Right Upper	Left Lower	Left Upper
Normoactive	Normoactive	Normoactive	Normoactive

Last Bowel Movement: 03/24/2026

Comments:

Constipation

- [ ] Chronic
- [ ] Acute
- [ ] Occasional
- [ ] Internal Hemorrhoids
- [ ] External Hemorrhoids
- [ ] Laxative/Enema Use

Comments:

Stool Appearance

- [x] Within Normal Limits
- [ ] Gray
- [ ] Tarry
- [ ] Black
- [ ] Fresh Blood
- [ ] Others (Specify):

Comments:

Genitourinary (Genitals & Urinary)

(M1600) Has this patient been treated for a Urinary Tract Infection in the past 14 days?

- [x] 0. No
- [ ] 1. Yes
- [ ] NA - Patient on prophylactic treatment
- [ ] UK - Unknown

(M1610) Urinary Incontinence or Urinary Catheter Presence:

- [ ] Patient is incontinent
  - [ ] Day time
  - [ ] Night time
  - [ ] Day and Night
  - [ ] Timed-voiding defers incontinence
  - [ ] Occasional stress incontinence
- [ ] Patient requires a urinary catheter (specifically: external, indwelling, intermittent, or suprapubic)
- [x] No incontinence or catheter (includes anuria or ostomy for urinary drainage)

Genitourinary Assessment

- [x] Within Normal Limits

Abnormalities:

- [ ] Frequency
- [ ] Dysuria
- [ ] Urgency
- [ ] Anuria
- [ ] Nocturia
- [ ] Oliguria
- [ ] Retention
- [ ] Hematuria
- [ ] Irrigation
- [ ] Incontinence
- [ ] Diapers Used
- [ ] Penile Discharge
- [ ] Vaginal Bleeding/Discharge

Others (Specify):

Urine Color:

- [x] Clear
- [ ] Transparent
- [x] Pale Yellow
- [ ] Brownish Orange
- [ ] Pinkish Red
- [ ] Dark Yellow
- [ ] Blue or Green
- [ ] Foamy
- [ ] Transparent Yellow

Urine Consistency:

- [ ] Hematuria
- [ ] Sediment
- [ ] Odorous
- [ ] Cloudy

Urinary Catheter:

- [ ] External Catheter
- [ ] Foley
- [ ] Suprapubic
- [ ] Nephrostomy

Last Catheter Change: mm/dd/yyyy

Fr:

cc:

Comments:

(M1306) Does this patient have at least one Unhealed Pressure Ulcer/Injury at Stage 2 or Higher or designated as Unstageable?
(Excludes Stage 1 pressure injuries and all healed pressure ulcers/injuries)

- [x] 0. No **[→ Skip to M1322, Current Number of Stage 1 Pressure Injuries at SOC/ROC; Skip to M1324, Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable at DC]**
- [ ] 1. Yes

(M1311) Current Number of Unhealed Pressure Ulcers/Injuries at Each Stage

A1. Stage 2: Partial thickness loss of dermis presenting as a shallow open ulcer with a red or pink wound bed, without slough. May also present as an intact or open/ruptured blister. **Number of Stage 2 pressure ulcers**	^
B1. Stage 3: Full thickness tissue loss. Subcutaneous fat may be visible but bone, tendon, or muscle is not exposed. Slough may be present but does not obscure the depth of tissue loss. May include undermining and tunneling. **Number of Stage 3 pressure ulcers**	^
C1. Stage 4: Full thickness tissue loss with exposed bone, tendon, or muscle. Slough or eschar may be present on some parts of the wound bed. Often includes undermining and tunneling. **Number of Stage 4 pressure ulcers**	^
D1. Unstageable: Non-removable dressing/device: Known but not stageable due to non¬ removable dressing/device **Number of unstageable pressure ulcers/injuries due to non-removable dressing/device**	^
E1. Unstageable: Slough and/or eschar: Known but not stageable due to coverage of wound bed by slough and/or eschar **Number of unstageable pressure ulcers due to coverage of wound bed by slough and/or eschar**	^
F1. Unstageable: Deep tissue injury Number of unstageable pressure injuries presenting as deep tissue injury	^

(M1322) Current Number of Stage 1 Pressure Injuries:
Intact skin with non-blanchable redness of a localized area usually over a bony prominence. Darkly pigmented skin may not have a visible blanching; in dark skin tones only it may appear with persistent blue or purple hues.
Stage 1 Pressure Injuries: **0**

(M1324) Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable:
(Excludes pressure ulcer/injury that cannot be staged due to a non-removable dressing/device, coverage of wound bed by slough and/or eschar, or deep tissue injury.)
Stage of Most Problematic: **Patient has no pressure ulcers or no stageable pressure ulcers**

(M1330) Does this patient have a Stasis Ulcer?

- [x] 0. No **[GO TO M1340]**
- [ ] 1. Yes, patient has BOTH observable and unobservable stasis ulcers
- [ ] 2. Yes, patient has observable stasis ulcers ONLY
- [ ] 3. Yes, patient has unobservable stasis ulcers ONLY (known but not observable due to non¬ removable dressing/device) **[GO TO M1340]**

(M1332) Current Number of Stasis Ulcer(s) that are Observable:

- [ ] 1. One
- [ ] 2. Two
- [ ] 3. Three
- [ ] 4. Four or more

(M1334) Status of Most Problematic Stasis Ulcer that is Observable

- [ ] 1. Fully granulating
- [ ] 2. Early/partial granulation
- [ ] 3. Not healing

(M1340) Does this patient have a Surgical Wound?

- [ ] 0. No **[GO TO N0415]**
- [ ] 1. Yes, patient has at least one observable surgical wound
- [x] 2. Surgical wound known but not observable due to non-removable dressing/device **[GO TO N0415]**

(M1342) Status of Most Problematic Surgical Wound that is Observable

- [ ] 0. Newly epithelialized
- [ ] 1. Fully granulating
- [ ] 2. Early/partial granulation
- [ ] 3. Not healing

Skin Color

- [x] Within Normal Limits
- [ ] Pale
- [ ] Jaundice
- [ ] Cyanotic

Skin Turgor:

- [x] Good
- [ ] Fair
- [ ] Poor

Skin Abnormalities:

- [x] Dry
- [x] Warm
- [ ] Cool
- [ ] Rash
- [ ] Ulcer
- [ ] Ostomy
- [ ] Incision
- [ ] Diaphoretic
- [ ] Others (Specify):

Additional Skin Comments

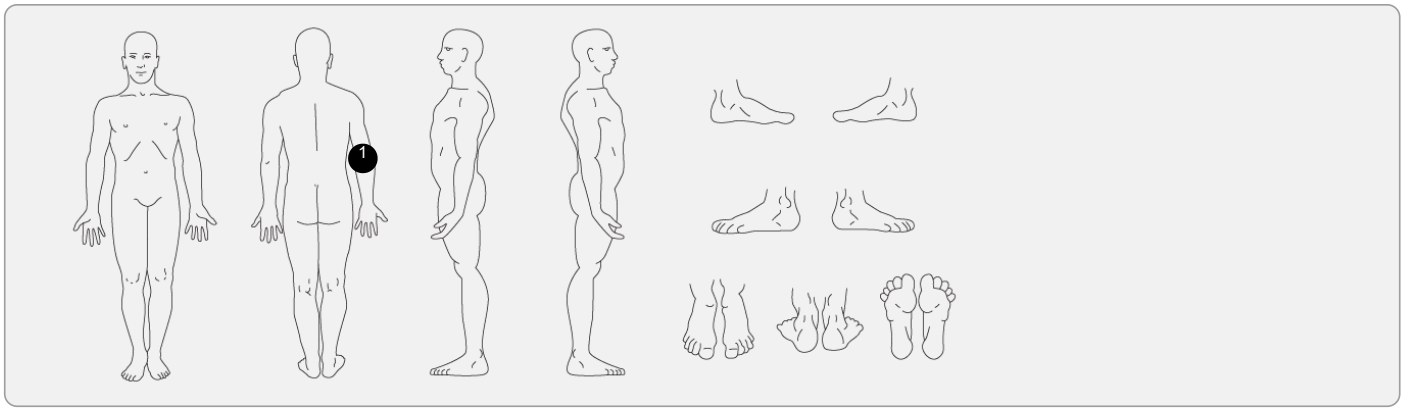
Norton Scale for Pressure Ulcer Risk Assessment

	Good (4)	Fair (3)	Poor (2)	Very Bad (1)
Physical Condition	(●) Good	( ) Fair	( ) Poor	( ) Very Bad
Mental Condition	(●) Alert	( ) Apathetic	( ) Confused	( ) Stuporous
Activity	( ) Ambulant	(●) Walks With Help	( ) Chairbound	( ) Bedfast
Mobility	( ) Full	(●) Slightly Impaired	( ) Very Limited	( ) Immobile
Incontinence	(●) None	( ) Occassional	( ) Usually Urinary	( ) Urinary and Fecal
			Total Score:	18

Wound

Wound Worksheet

Fetch Wounds From Previous Wound Note



Wound 1

Location	Size	Pressure Injury Stage
Right elbow.	**0 x 0 x 0**	-
Type	Drainage	Amount Of Drainage
-	-	-
Odor	Color	Tunneling/Undermining
-	-	-
Tunneling Location	Tunneling Length (cm)	Undermining Location
-	-	-
Undermining Length (cm)	Surrounding Tissue	Wound Base
-	-	-
Level Of Pain	Edema	Stoma
-	-	-
Slough and/or Echar	Slough (%)	Eschar (%)
-	-	-
Granulation (%)	Pink/Red (%)	Necrotic (%)
-	-	-
Wound Status	Wound Description	Satisfaction of Return Demonstration
-	-	- -
Patient's Response to Wound Care	Wound Care	
- - undefined	**Surgical wound not observable due to non-removable dressing.**	
Onset Date		
2026-03-10		

SAFETY & RISK ASSESSMENT

Home Safety Assessment

Home Environment

☑ **Architectural Barriers**	☐ **No fire extinguisher**	☐ **No emergency exit plan**
☐ **Medications are improperly stored**	☐ **Inadequate electricity / cooling / heating**	☐ **Rodent / insect infestation**
☐ **No telephone**	☐ **Inadequate supply of water**	☐ **Unsafe electrical wiring**
☑ **Cluttered surroundings**	☐ **Inadequate sanitation / plumbing**	☐ **Nonworking stove / refrigerator**
☐ **Inadequate railing**	☐ **Inadequate smoke detectors on all levels of home**	☐ **Exposed flammables**
☐ **Unsafe oxygen use**	☑ **Unsafe storage of equipments**	☐ **Inadequate Lighting**

Other:

Type of Assistance Patient Receives - other than from Home Health Agency Staff

ADL(Bathing, dressing, toileting, eating/feeding, bowel)	☑ **Family/Friends**	☐ **Provider Services**	☐ **Paid Caregiver**	☐ **Volunteer Organizations**
IADL (Meds, meals, house-keeping, laundry, telephone, shipping, finances)	☑ **Family/Friends**	☐ **Provider Services**	☐ **Paid Caregiver**	☐ **Volunteer Organizations**
Psychosocial Support	☑ **Family/Friends**	☐ **Provider Services**	☐ **Paid Caregiver**	☐ **Volunteer Organizations**
Assistance with doctors appointment, delivery of medications	☑ **Family/Friends**	☐ **Provider Services**	☐ **Paid Caregiver**	☐ **Volunteer Organizations**
Management of Finances	☑ **Family/Friends**	☐ **Provider Services**	☐ **Paid Caregiver**	☐ **Volunteer Organizations**

Comments:

(M2102) Types and Sources of Assistance

Determine the ability and willingness of non-agency caregivers (such as family members, friends, or privately paid caregivers) to provide assistance for the following activities, if assistance is needed. Excludes all care by your agency staff

F. Supervision and Safety (for example, due to cognitive impairment)

◯ **0. No assistance needed – patient is independent or does not have needs in this area**

- ◯ 1. Non-agency caregiver(s) currently provide assistance
- ☑ 2. Non-agency caregiver(s) need training/supportive services to provide assistance
- ◯ 3. Non-agency caregiver(s) are not likely to provide assistance OR it is unclear if they will provide assistance
- ◯ 4. Assistance needed, but no non-agency caregiver(s) available

Safety Hazards Affecting Patient

- ☑ Stairs
- ☐ Inadequate lighting, heating and cooling
- ☐ Insect/Rodent Infestation
- ☐ No gas/electric appliance
- ☐ No hazards identified
- ☐ No running water, plumbing
- ☑ Narrow or obstructed walkway
- ☐ Lack of fire safety devices
- ☑ Cluttered/soiled living area

Others:

Safety Measures

- ◯ 1 - Bleeding precautions
- ◯ 2 - O_2 precautions
- ◯ 3 - Seizure precautions
- ☑ 4 - Fall precautions
- ◯ 5 - Aspiration precautions
- ◯ 6 - Siderails up
- ◯ 7 - Elevate head of bed
- ◯ 8 - 24 hr. supervision
- ☑ 9- Clear Pathways up
- ◯ 10 - Lock w/c with transfers
- ◯ 11 - Infection control measures
- ◯ 12 - Walker/cane
- ◯ 13 - Sharps Precaution
- ☑ 14 - Other:

911/ER protocol, Universal precaution

Hospitalization Risk Assessment

Prior Pattern: Check all that apply

- ☑ > 1 Hospitalizations or ER visits in the past 12 months
- ☐ History of falls

Chronic conditions: Check all that apply (M1020/1022/1024)

- ☐ HF
- ☐ Diabetes
- ☐ COPD
- ☐ Chronic skin ulcers (Wound consult if indicated for any wounds)
- ☐ HIV/AIDS

Risk factors: (Check all that apply)

- ☑ Discharged from hospital or skilled nursing facility
- ☑ More than 2 secondary diagnoses
- ☐ Low socioeconomic status or financial concerns ♦
- ☐ Lives alone ►♦
- ☐ Inadequate support network ♦
- ☑ ADL assistance needed ►
- ☑ Home safety risks ► ♦
- ☐ Dyspnea ► ★
- ☐ Help with managing medications needed ► ★
- ☐ Non-compliance with medication regimen ♦ ★
- ☐ Confusion ♦ ★
- ☐ Pressure ulcer ★
- ☐ Stasis ulcer ★
- ☐ Overall Poor Status/Prognosis ■
- ☐ Low literacy level ♦
- ☐ Depression ♦
- ☐ ►Consider Therapy referral (PT, OT, ST)
- ☑ ♦Consider MSW referral
- ☐ ■ Consider Hospice referral
- ☐ ★Consider RN referral, if not ordered

Total number of checked boxes is:
6

Your agency may want to select a threshold score to target patients at high risk.

(For example: 5 or greater risk factors may indicate that the patient is at risk for hospitalization. Note: This number is for convenience only and has not been tested or validated. The agency may modify the score based upon the needs of their patient population.)

Carry out patient specific interventions as appropriate/ordered, if patient is at risk for hospitalization: (Coordinate with M2250)

Referrals:

- ☑ SN
- ☑ PT
- ☐ OT
- ☐ ST
- ☐ MSW
- ☐ HHA
- ☐ Dietary Consultant

Other:

Consider implementing any of the following interventions, if patient is at risk for hospitalization:

- ☑ Medication Management
- ☑ Medication Reconciliation
- ☑ Assess patient's: knowledge, ability, resources and adherence
- ☑ Education
- ☑ Patient/family education
- ☐ Enrollment into a disease management program

(Specify):

- ☐ Hospice/Palliative Referral
- ☑ Individualized Patient Emergency Care Plan
- ☑ Fall Prevention Program
- ☐ Phone Monitoring
- ☐ Telemonitoring
- ☐ Wound Care
- ☐ Front-loading Visits
- ☑ Care Coordination (physicians, hospitals, nursing homes ...)

Immunizations (M1040, M1045, M1050, M1055)

- ☐ Influenza
- ☐ Pneumococcal

Fall Prevention Program:

Other:

(M1033) Risk for Hospitalization: Which of the following signs or symptoms characterize this patient as at risk for hospitalization? (Mark all that apply.)

- ☐ 1. History of falls (2 or more falls – or any fall with an injury – in the past 12 months)
- ☐ 2. Unintentional weight loss of a total of 10 pounds or more in the past 12 months
- ☐ 3. Multiple hospitalizations (2 or more) in the past 6 months
- ☐ 4. Multiple emergency department visits (2 or more) in the past 6 months
- ☐ 5. Decline in mental, emotional, or behavioral status in the past 3 months
- ☐ 6. Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months
- ☐ 7. Currently taking 5 or more medications
- ☐ 8. Currently reports exhaustion

☐ **9. Other risk(s) not listed in 1 - 8**

☑ **10. None of the above**

Additional Risk Information:

Prior Hospitalization

	Number of times:
● Yes ○ No	1

FALL RISK ASSESSMENT TOOL

MAHC 10* - FALL RISK ASSESSMENT TOOL

	Yes	No
Age 65+	○	✓
Diagnosis (3 or more co-existing) Includes only documented medical diagnosis	✓	○
Prior history of falls within 3 months An unintentional change in position resulting in coming to rest on the ground or at a lower level	○	✓
Incontinence Inability to make it to the bathroom or commode in timely manner. Includes frequency, urgency, and/or nocturia.	○	✓
Visual impairment Includes but not limited to, macular degeneration, diabetic retinopathies, visual field loss, age related changes, decline in visual acuity, accommodation, glare tolerance, depth perception, and night vision or not wearing prescribed glasses or having the correct prescription.	✓	○
Impaired functional mobility May include patients who need help with IADLS or ADLS or have gait or transfer problems, arthritis, pain, fear of falling, foot problems, impaired sensation, impaired coordination or improper use of assistive devices.	✓	○
Environmental hazards May include but not limited to, poor illumination, equipment tubing, inappropriate footwear, pets, hard to reach items, floor surfaces that are uneven or cluttered, or outdoor entry and exits.	✓	○
Poly Pharmacy (4 or more prescriptions - any type) All PRESCRIPTIONS including prescriptions for OTC meds. Drugs highly associated with fall risk include but not limited to, sedatives, antidepressants, tranquilizers, narcotics, antihypertensives, cardiac meds, corticosteroids, anti-anxiety drugs, anticholinergic drugs, and hypoglycemic drugs.	○	✓
Pain affecting level of function Pain often affects an individual's desire or ability to move or pain can be a factor in depression or compliance with safety recommendations.	✓	○
Cognitive impairment Could include patients with dementia, Alzheimer's or stroke patients or patients who are confused, use poor judgment, have decreased comprehension, impulsivity, memory deficits. Consider patients ability to adhere to the plan of care.	○	✓
A score of 4 or more is considered at risk for falling	5	

Fire Assessment for Patient with Oxygen

☑ **Patient is not using oxygen**

Does patient have No Smoking signs posted?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Does patient or anyone in the home smoke with oxygen in use?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Are smoke detectors present and working properly?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Does patient have a properly functioning fire extinguisher?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Are oxygen cylinders stored properly?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Are all electrical cords near oxygen intact and free for fraying?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Does patient have an evacuation plan in case of fire?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Are all cleaning fluids and aerosols stored away from oxygen, and not used when oxygen is in use?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Does patient refrain from using petroleum products around oxygen?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Does patient only used water-based body and lip moisturizers?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No

Comments:

FUNCTIONAL ASSESSMENT (SELF CARE)

(GG0100) Prior Functioning: Everyday Activities: Indicate the patient's usual ability with everyday activities prior to the current illness, exacerbation, or injury.

- **3. Independent** – Patient completed the activities by him/herself, with or without an assistive device, with no assistance from a helper
- **2. Needed Some Help** – Patient needed partial assistance from another person to complete activities
- **1. Dependent** – A helper completed the activities for the patient.
- **8. Unknown**
- **9. Not Applicable**

	Independent	Needed Some Help	Dependent	Unknown	Not Applicable
A. Self Care: Code the patient's need for assistance with bathing, dressing, using the toilet, or eating prior to the current illness, exacerbation, or injur	○	✓	○	○	○
B. Indoor Mobility (Ambulation): Code the patient's need for assistance with walking from room to room (with or without a device such as cane, crutch or walker) prior to the current illness, exacerbation, or injury.	○	✓	○	○	○
C. Stairs: Code the patient's need for assistance with internal or external stairs (with or without a device such as cane, crutch, or walker) prior to the current illness, exacerbation or injury.	○	✓	○	○	○
D. Functional Cognition: Code the patient's need for assistance with planning regular tasks, such as shopping or remembering to take medication prior to the current illness, exacerbation, or injury.	○	✓	○	○	○

(GG0110) Prior Device Use

Indicate devices and aids used by the patient prior to the current illness, exacerbation, or injury (**Check all that apply)**

- [ ] **A. Manual wheelchair**
- [ ] **B. Motorized wheelchair and/or scooter**
- [ ] **C. Mechanical lift**
- [ ] **D. Walker**
- [ ] **E. Orthotics/Prosthetics**
- [x] **Z. None of the above**

(GG0130) Self Care

(GG0130) SOC/ROC Performance

Code the patient's usual performance at SOC/ROC for each activity using the 6-point scale. If activity was not attempted at SOC/ROC, code the reason.
Safety and Quality of Performance – If helper assistance is required because patient's performance is unsafe or of poor quality, score according to amount of assistance provided.
Activities may be completed with or without assistive devices.

06. Independent – Patient completes the activity by him/herself with no assistance from a helper.
05. Setup or clean-up assistance – Helper sets up or cleans up; patient completes activity. Helper assists only prior to or following the activity.
04. Supervision or touching assistance – Helper provides verbal cues and/or touching/steadying and/or contact guard assistance as patient completes activity. Assistance may be provided throughout the activity or intermittently.
03. Partial/moderate assistance – Helper does LESS THAN HALF the effort. Helper lifts, holds or supports trunk or limbs, but provides less than half the effort.
02. Substantial/maximal assistance – Helper does MORE THAN HALF the effort. Helper lifts or holds trunk or limbs and provides more than half the effort.
01. Dependent – Helper does ALL of the effort. Patient does none of the effort to complete the activity. Or, the assistance of 2 or more helpers is required for the patient to complete the activity.

If activity was not attempted, code reason:

07. Patient refused
09. Not applicable – Not attempted and the patient did not perform this activity prior to the current illness, exacerbation or injury.
10. Not attempted due to environmental limitations (e.g., lack of equipment, weather constraints)
88. Not attempted due to medical conditions or safety concerns

A. Eating

- [ ] (06) Independent
- [x] (05) Setup or clean-up assistance
- [ ] (04) Supervision or touching assistance
- [ ] (03) Partial/moderate assistance
- [ ] (02) Substantial/maximal assistance
- [ ] (01) Dependent
- [ ] (07) Patient refused
- [ ] (09) Not applicable
- [ ] (10) Not attempted due to environmental limitations
- [ ] (88) Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

B. Oral Hygiene

- [ ] (06) Independent
- [x] (05) Setup or clean-up assistance
- [ ] (04) Supervision or touching assistance
- [ ] (03) Partial/moderate assistance
- [ ] (02) Substantial/maximal assistance
- [ ] (01) Dependent
- [ ] (07) Patient refused
- [ ] (09) Not applicable
- [ ] (10) Not attempted due to environmental limitations
- [ ] (88) Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

C. Toileting Hygiene

- [ ] (06) Independent
- [x] (05) Setup or clean-up assistance
- [ ] (04) Supervision or touching assistance
- [ ] (03) Partial/moderate assistance
- [ ] (02) Substantial/maximal assistance
- [ ] (01) Dependent
- [ ] (07) Patient refused
- [ ] (09) Not applicable
- [ ] (10) Not attempted due to environmental limitations
- [ ] (88) Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

E. Shower/Bathe Self

- [ ] (06) Independent
- [x] (05) Setup or clean-up assistance
- [ ] (04) Supervision or touching assistance
- [ ] (03) Partial/moderate assistance
- [ ] (02) Substantial/maximal assistance
- [ ] (01) Dependent
- [ ] (07) Patient refused
- [ ] (09) Not applicable
- [ ] (10) Not attempted due to environmental limitations
- [ ] (88) Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

F. Upper Body Dressing

- [ ] (06) Independent
- [x] (05) Setup or clean-up assistance
- [ ] (04) Supervision or touching assistance
- [ ] (03) Partial/moderate assistance
- [ ] (02) Substantial/maximal assistance
- [ ] (01) Dependent
- [ ] (07) Patient refused
- [ ] (09) Not applicable
- [ ] (10) Not attempted due to environmental limitations
- [ ] (88) Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

G. Lower Body Dressing

- [ ] (06) Independent
- [x] (05) Setup or clean-up assistance
- [ ] (04) Supervision or touching assistance
- [ ] (03) Partial/moderate assistance
- [ ] (02) Substantial/maximal assistance
- [ ] (01) Dependent
- [ ] (07) Patient refused
- [ ] (09) Not applicable
- [ ] (10) Not attempted due to environmental limitations
- [ ] (88) Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

H. Putting on/Taking Off Footwear

- [ ] (06) Independent
- [x] (05) Setup or clean-up assistance
- [ ] (04) Supervision or touching assistance
- [ ] (03) Partial/moderate assistance
- [ ] (02) Substantial/maximal assistance
- [ ] (01) Dependent
- [ ] (07) Patient refused
- [ ] (09) Not applicable
- [ ] (10) Not attempted due to environmental limitations
- [ ] (88) Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(M1800) Grooming

Current ability to tend safely to personal hygiene needs (specifically: washing face and hands, hair care, shaving or make up, teeth or denture care, or fingernail care)

- [ ] **0. Able to groom self unaided, with or without the use of assistive devices or adapted methods.**
- [x] **1. Grooming utensils must be placed within reach before able to complete grooming activities.**
- [ ] **2. Someone must assist the patient to groom self.**
- [ ] **3. Patient depends entirely upon someone else for grooming needs**

(M1810) Ability to Dress Upper Body

Current **Ability to Dress Upper Body** safely (with or without dressing aids) including undergarments, pullovers, front-opening shirts and blouses, managing zippers, buttons, and snaps:

- [ ] **0. Able to get clothes out of closets and drawers, put them on and remove them from the upper body without assistance.**
- [x] **1. Able to dress upper body without assistance if clothing is laid out or handed to the patient.**
- [ ] **2. Someone must help the patient put on upper body clothing.**
- [ ] **3. Patient depends entirely upon another person to dress the upper body.**

(M1820) Ability to Dress Lower Body

Current **Ability to Dress Lower Body** safely (with or without dressing aids) including undergarments, slacks, socks or nylons, shoes:

- [ ] **0. Able to obtain, put on, and remove clothing and shoes without assistance.**
- [x] **1. Able to dress lower body without assistance if clothing and shoes are laid out or handed to the patient.**
- [ ] **2. Someone must help the patient put on undergarments, slacks, socks or nylons, and shoes.**
- [ ] **3. Patient depends entirely upon another person to dress lower body.**

(M1830) Bathing

Current ability to wash entire body safely. **Excludes grooming (washing face, washing hands, and shampooing hair).**

- [ ] **0. Able to bathe self in shower or tub independently, including getting in and out of tub/shower.**
- [ ] **1. With the use of devices, is able to bathe self in shower or tub independently, including getting in and out of the tub/shower.**

- [x] 2. Able to bathe in shower or tub with the intermittent assistance of another person: (a) for intermittent supervision or encouragement or reminders, OR (b) to get in and out of the shower or tub, OR (c) for washing difficult to reach areas.
- [ ] 3. Able to participate in bathing self in shower or tub, but requires presence of another person throughout the bath for assistance or supervision.
- [ ] 4. Unable to use the shower or tub, but able to bathe self independently with or without the use of devices at the sink, in chair, or on commode.
- [ ] 5. Unable to use the shower or tub, but able to participate in bathing self in bed, at the sink, in bedside chair, or on commode, with the assistance or supervision of another person
- [ ] 6. Unable to participate effectively in bathing and is bathed totally by another person.

(M1840) Toilet Transferring

Current ability to get to and from the toilet or bedside commode safely and transfer on and off toilet/commode

- [ ] 0. Able to get to and from the toilet and transfer independently with or without a device
- [x] 1. When reminded, assisted, or supervised by another person, able to get to and from the toilet and transfer.
- [ ] 2. Unable to get to and from the toilet but is able to use a bedside commode (with or without assistance).
- [ ] 3. Unable to get to and from the toilet or bedside commode but is able to use a bedpan/urinal independently
- [ ] 4. Is totally dependent in toileting.

(M1845) Toileting Hygiene

Current ability to maintain perineal hygiene safely, adjust clothes and/or incontinence pads before and after using toilet, commode, bedpan, urinal. If managing ostomy, includes cleaning area around stoma, but not managing equipment.

- [ ] 0. Able to manage toileting hygiene and clothing management without assistance.
- [x] 1. Able to manage toileting hygiene and clothing management without assistance if supplies/implements are laid out for the patient.
- [ ] 2. Someone must help the patient to maintain toileting hygiene and/or adjust clothing.
- [ ] 3. Patient depends entirely upon another person to maintain toileting hygiene

(M1850) Transferring

Current ability to move safely from bed to chair, or ability to turn and position self in bed if patient is bedfast

- [ ] 0. Able to independently transfer.
- [x] 1. Able to transfer with minimal human assistance or with use of an assistive device.
- [ ] 2. Able to bear weight and pivot during the transfer process but unable to transfer self.
- [ ] 3. Unable to transfer self and is unable to bear weight or pivot when transferred by another person
- [ ] 4. Bedfast, unable to transfer but is able to turn and position self in bed.
- [ ] 5. Bedfast, unable to transfer and is unable to turn and position self

(M1860) Ambulation/Locomotion

Current ability to walk safely, once in a standing position, or use a wheelchair, once in a seated position, on a variety of surfaces.

- [ ] 0. Able to independently walk on even and uneven surfaces and negotiate stairs with or without railings (specifically: needs no human assistance or assistive device).
- [ ] 1. With the use of a one-handed device (for example, cane, single crutch, hemi-walker), able to independently walk on even and uneven surfaces and negotiate stairs with or without railings.
- [ ] 2. Requires use of a two-handed device (for example, walker or crutches) to walk alone on a level surface and/or requires human supervision or assistance to negotiate stairs or steps or uneven surfaces.
- [x] 3. Able to walk only with the supervision or assistance of another person at all times.
- [ ] 4. Chairfast, unable to ambulate but is able to wheel self independently.
- [ ] 5. Chairfast, unable to ambulate and is unable to wheel self
- [ ] 6. Bedfast, unable to ambulate or be up in a chair.

FUNCTIONAL ASSESSMENT (MOBILITY & MUSCULOSKELETAL)

(GG0170) Mobility

(GG0170A) A. Roll left and right: The ability to roll from lying on back to left and right side, and return to lying on back on the bed.

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170B) B. Sit to lying: The ability to move from sitting on side of bed to lying flat on the bed.

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170C) C. Lying to sitting on side of bed: The ability to move from lying on the back to sitting on the side of the bed with feet flat on the floor, and with no back support.

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170D) D. Sit to stand: The ability to come to a standing position from sitting in a chair, wheelchair, or on the side of the bed.

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170E) E. Chair/bed-to-chair transfer: The ability to transfer to and from a bed to a chair (or wheelchair).

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170F) F. Toilet transfer: The ability to get on and off a toilet or commode.

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [ ] 07. Patient refused
- [ ] 09. Not applicable

- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170G) G. Car transfer: The ability to transfer in and out of a car or van on the passenger side. Does not include the ability to open/close door or fasten seat belt.

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170I) I. Walk 10 feet: Once standing, the ability to walk at least 10 feet in a room, corridor, or similar space. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170M, 1 step (curb)*

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170J) J. Walk 50 feet with two turns: Once standing, the ability to walk 50 feet and make two turns.

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170K) K. Walk 150 feet: Once standing, the ability to walk at least 150 feet in a corridor or similar space.

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170L) L. Walking 10 feet on uneven surfaces: The ability to walk 10 feet on uneven or sloping surfaces (indoor or outdoor), such as turf or gravel.

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170M) M. 1 step (curb): The ability to go up and down a curb and/or up and down one step. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.*

SOC/ROC Performance

- [ ] 06. Independent
- [x] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170N) N. 4 steps: The ability to go up and down four steps with or without a rail. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.*

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170O) O. 12 steps: The ability to go up and down 12 steps with or without a rail.

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170P) P. Picking up object: The ability to bend/stoop from a standing position to pick up a small object, such as a spoon, from the floor.

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170Q) Q. Does patient use a wheelchair and/or scooter?

- [x] 0. No **[GOTO M1600]**
- [ ] 1. Yes
- [ ] Not assessed (no information)

(GG0170R) R. Wheel 50 feet with two turns: Once seated in wheelchair/scooter, the ability to wheel at least 50 feet and make two turns.

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns

- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ (-) Not assessed/no information

(GG0170RR) RR1. Indicate the type of wheelchair or scooter used.
SOC/ROC Performance

- ◯ 1. Manual
- ◯ 2. Motorized
- ◯ Not assessed (no information)

(GG0170S) S. Wheel 150 feet: Once seated in wheelchair/scooter, the ability to wheel at least 150 feet in a corridor or similar space.
SOC/ROC Performance

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170SS) SS1. Indicate the type of wheelchair or scooter used.
SOC/ROC Performance

- ◯ 1. Manual
- ◯ 2. Motorized
- ◯ Not assessed (no information)

Gait and Mobility Abnormalities

- ☐ No Problems Identified
- ☐ Weakness
- ☐ Poor Balance
- ☐ Bed Bound
- ☐ Chair Bound
- ☐ Joint Stiffness
- ☐ Shuffling/Wide-based Gait
- ☐ Orthopedic Aftercare
- ☑ High Risk for Falls
- ☑ Weight Bearing Restriction
- ☑ Poor Conditioning
- ☐ Fracture:
- ☐ Joints Pain:
- ☐ Amputation:
- ☐ Atrophy:
- ☐ Paresthesia:
- ☐ Contractures: Site
- ☐ Foot Drop: ☐ Right ☐ Left

Fall(s) since last visit (Specify):

Weakness (Specify):

Change in ADL (Explain):

Comments:

Range of Motion

- ☑ Limited Range of Motion: ☑ RUE ☐ RLE ☐ LUE ☐ LLE
- ☐ Quadriplegia ☐ Hemiplegia ☐ Paraplegia

Comments:

Grip Strength

- ☐ Normal ☐ Strong ☑ Weak ☐ Equal
- ☑ Unequal ☐ Bilateral ☑ Right ☐ Left

Others (Specify):

Grasp Strength

- ☐ Normal ☐ Strong ☑ Weak ☐ Equal ☑ Unequal

Comments:

DME and Supplies

DME Lookup

#18B ACTIVITIES PERMITTED

- ◯ 1- Complete bedrest
- ◯ 2 - Bedrest/BRP
- ◯ 3 - Up as tolerated
- ◯ 4 - Transfer bed/chair
- ◯ 5 - Exercises prescribed
- ◯ 6 - Partial weight bearing
- ◯ 7 - Independent in home
- ◯ 8 - Crutches
- ◯ 9 - Cane
- ◯ A - Wheelchair
- ◯ B - Walker
- ◯ C - No restrictions
- ◯ D - Other (specify)

ENDOCRINE (DIABETIC MANAGEMENT)

Endocrine

Is patient diabetic?	◯ Yes	◯ No	
Insulin dependent?	◯ Yes	◯ No	How Long?
Is patient independently able to draw up correct dose of insulin?	◯ Yes	◯ No	◯ N/A
Is patient able to administer own insulin?	◯ Yes	◯ No	◯ N/A
Is patient taking oral hypoglycemic agent?	◯ Yes	◯ No	◯ N/A
Is patient independent with glucometer use?	◯ Yes	◯ No	◯ N/A
Is caregiver able to draw up and administer insulin?	◯ Yes	◯ No	◯ N/A
Is caregiver independent with glucometer use?	◯ Yes	◯ No	◯ N/A
Does patient or caregiver routinely perform inspection of the patient's lower extremities?	◯ Yes	◯ No	◯ N/A

Diabetic Management

☐ Diet ☐ Hypoglycemic ☐ Exercise ☐ Insulin

Insulin Administered By:

☐ N/A ☐ Caregivier ☐ Patient ☐ Skilled Nurse

S&S of Hyperglycemia

☐ Polyuria ☐ Polyphagia ☐ Polydipsia ☐ Blurred Vision ☐ Fatigue

Others (Specify):

S&S of Hypoglycemia

☐ Perspiration ☐ Dizziness ☐ Irritability ☐ Pale ☐ Shakiness

Others (Specify):

Blood glucose range: **Low to High (mg/dL):**

Comments:

PLAN OF CARE AND SKILLED INTERVENTIONS

Plan of Care Orders

Orders and Treatment (Specify Frequency, Duration and Purpose of Visits): §484.60 (2.iv)

SN Frequency: 1/ wk x 4 wks **MSW Frequency:** **PT Frequency: 1/ wk x 1 wk, 2/ wks x 3 wks** **OT Frequency:**

HHA Frequency: **ST Frequency** **Dietitian Frequency:**

MD SIGNATURE COVERS ALL PAGES
ADMIT PATIENT TO MEADOWS HOME HEALTH LLC. ALL DIAGNOSIS AND MEDICATIONS VERIFIED WITH THE PRIMARY CARE PHYSICIAN.

Patient is a 38-year-old male admitted to Meadows Home Health. The patient is considered homebound because the patient has recent displaced fractures of the head and neck of the right radius; with recent surgery and a non-removable surgical dressing to the right elbow and requires the assistance of another person to safely leave the home. Objective findings supporting homebound status include postoperative right elbow wound not observable due to non-removable dressing, frequent activity-provoked pain of the right elbow/shoulder (pain 3/10 with activity), documented need to maintain limb rest and support during movement, and a documented fall risk (MAHC score 5) while the patient lives alone. Leaving home would require a considerable and taxing effort because transfers, car entry/exit, negotiating steps/curbs or uneven surfaces, and maintaining wound/dressing integrity would provoke pain, risk wound disruption or infection, and exceed the patient's current functional capacity without human assistance; skilled staff documented the patient "needs assistance to leave the place of residence." Allowable absences are limited to brief, medically necessary outings (for example, physician or wound care/therapy appointments); extended or routine community outings are unsafe at this time. These objectively documented functional limitations, postoperative wound care needs, pain and impaired mobility demonstrate that the need for medical necessity for skilled home health services (skilled nursing for wound/pain management and safety monitoring, and skilled therapy for mobility and transfer training) to support safe recovery in the home setting.

Frequency and duration:
HH SN frequency: 1 time a week for 4 weeks for skilled observation and assessment, medication reconciliation, pain assessment, infection control measures, diet, home safety and disease process education.
HH PT frequency: 1 time a week for 1 week, 2 times a week for 3 weeks evaluation and treatment for mobility, gait, transfer, establish therapeutic exercises, strengthening, home safety and fall prevention techniques training effective on 03/27/2026.

Goals:
-Reduce patient-reported pain at the right elbow/shoulder to ≤2/10 at rest and with activity using current analgesic regimen and prescribed nonpharmacologic measures (ice, positioning, relaxation) as reported by the patient, within 4 weeks.
-Demonstrate improved functional mobility by achieving sit-to-stand transfers with supervision or minimal assistance and ambulating at least 50 feet with the prescribed assistive device (or wheeling 50 feet if using wheelchair) as observed by PT or RN, within 4 weeks.
-Patient and/or caregiver will correctly perform wound care procedures (including hand hygiene, dressing management, and infection-warning signs) and complete a return demonstration with ≥80% accuracy, within 4 weeks.
-Increase independence with self-care: patient will complete upper-body dressing with setup assistance only and manage toileting hygiene with supervision or setup assistance, as measured by RN/PT observation, within 4 weeks.
-Patient and/or caregiver will accurately demonstrate medication management (name, dose, frequency, indications, and when to call clinician) and complete medication reconciliation with RN documentation, within 4 weeks.

Teachings/Interventions

Skilled Intervention

Assessment/Instruction/Performance:
Skilled Nurse instructed the following: Explained Home Health service to patient/family/caregiver(s). Explained consent to care, release of liability, and patient rights & responsibilities. Consent to care, a liability release, and consent to photograph were signed by the patient/responsible party. A comprehensive assessment on all body systems with OASIS elements evaluation was initiated. Reconciled facility discharge medications against routine home prescriptions without issue. Discussed advance directives, privacy policy, COVID-19 prevention, 911 protocols, basic home safety, and fall prevention principles with patient/family/caregiver(s). Discussed plan of care draft with patient and explained what to expect in the next visits and the remainder of the care episode. Also explained that the initial discussion on the management of other disease processes will be done on the next visit.

Skilled Nurse assessed the patient's mobility and functional status related to fracture of head of right radius and neck of right radius. Skilled Nurse assessed the environment for safety at each visit to reduce the risk of falls or accidents. Skilled Nurse instructed the patient/caregiver on the importance of monitoring conditions and risk factors that could contribute to falls, such as fatigue and muscle weakness. Skilled Nurse assessed the patient's skin for breakdown at every visit due to decreased mobility associated with the fracture. Skilled Nurse assessed peripheral circulation, sensation, and motor function, especially in the affected extremity. Skilled Nurse encouraged the patient to mobilize as tolerated to promote blood circulation. Skilled Nurse instructed the patient/caregiver to maintain limb rest as indicated and to provide support to the joints above and below the fracture site, particularly during movement and repositioning. Skilled Nurse instructed the patient/caregiver on infection control techniques, including proper handwashing, use of gloves, and maintaining good personal hygiene. Skilled Nurse taught the patient/caregiver fall prevention strategies, including the proper use of assistive devices and keeping personal items within arm's reach. Skilled Nurse reinforced the importance of avoiding tasks beyond the patient's current functional ability and requesting assistance when needed.

Skilled nurse assessed the patient's pain level using an appropriate pain scale and noted that the patient experienced pain rated at 3/10 during the assessment. The nurse evaluated the effectiveness of current pain management strategies and instructed the patient to adhere to and take pain medication before pain becomes severe to achieve optimal pain control. Provided teachings on nonpharmacologic pain relief measures such as relaxation techniques and positioning. Instructed patient and caregiver to notify the skilled nurse or medical doctor if the patient experiences pain greater than 7/10, is unable to tolerate pain, or if pain is affecting the ability to perform normal activities and ADLs. The skilled nurse guided the patient in performing gentle movements and exercises within tolerance to enhance circulation and prevent stiffness, which aided in pain reduction. Following interventions, the patient reported a reduction in pain level and demonstrated improved comfort.

Skilled Nurse assessed patient's safety in environment, mobility and functional limitations to reduce falls or accidents every visit. Assessed the patient's ability to perform ADLs effectively and safely every visit. Instructed Patient/Caregiver on the importance of assessing conditions/risks that may contribute to occurrence of falls such as fatigue/ muscle weakness. Encouraged to perform exercises if tolerated for strengthening and maintenance of posture and flexibility. Home exercise program (HEP) was established, and patient/caregiver were educated on passive and active assistive range-of-motion (ROM) exercises. Skilled Nurse guided patient through supervised balance and gait training exercises to improve stability and coordination, as well as facilitating functional mobility tasks such as sit-to-stand transfers and safe ambulation techniques.

Skilled Nurse thoroughly conducted medication reconciliation.

☑ **Response to Teaching/Interventions**

Verbalized Understanding ☑ **Patient (%):** 70% ☑ **Caregiver (%):** 80%

Return Demonstration ☑ **Patient (%):** 70% ☑ **Caregiver (%):** 80%

Require Further Teaching ☑ **Patient** ☑ **Caregiver**

Comments:
The patient and caregiver demonstrated partial understanding of the teaching and interventions provided, as evidenced by their ability to accurately verbalize 3 out of 5 key interventions related to infection control, pain management, home safety, and fall prevention. They correctly repeated instructions on maintaining a moist wound environment and recognizing signs of infection such as increased redness, swelling, foul odor, drainage, or fever. Both also demonstrated appropriate return demonstration of pain management strategies including medication adherence, timing of analgesics, and non-pharmacologic measures such as positioning and gentle exercises within tolerance. The patient verbalized understanding of the importance of using assistive devices properly and the need for human assistance during transfers and ambulation. Caregiver participation was actively observed, confirming comprehension and readiness to assist with the patient's daily care, medication management, and safety precautions. Both patient and caregiver showed capability in recognizing when to report worsening symptoms to healthcare providers, including pain greater than 7/10, inability to tolerate pain, or signs of infection.

Instructions during visit

- [x] Assess/Observe
- [ ] Injection Administration
- [ ] Catheter change
- [ ] IV Therapy
- [ ] Wound Care
- [x] Teaching/Instructions

Patient Strength

- [x] Motivated Learner
- [x] Strong Support System
- [ ] Absence of Multiple Diagnosis
- [ ] Enhanced Socioeconomic Status

Other:

Conclusions

- [x] Skilled Intervention Needed
- [x] Skilled Instruction Needed
- [ ] No Skilled Service Needed

Other:
In home PT.

Rehabilitation Potential

- [ ] Rehabilitation potential is poor
- [ ] Rehabilitation potential is good
- [x] Rehabilitation potential is fair
- [ ] Rehabilitation potential is excellent

Others:

Conference With

- [x] MD
- [x] SN
- [x] PT
- [ ] OT
- [ ] ST
- [ ] MSW
- [ ] HHA
- [x] DON/DOPCS
- [x] Clinical Coordinator
- [x] Clinical Manager

Name:
Maria Bautista, RN CM

Regarding:
Patient status during start of care visit

Physician Contacted Re:
Patient status during start of care visit

Order Changes:
None

Plan for Next Visit:
Will continue to assess and evaluate patient understanding, functional status, and knowledge of self-care management on the next visit.

Next Physician Visit:
mm/dd/yyyy

Patient and caregiver education and training to facilitate timely discharge: §484.60 (2.xiii)
Ongoing education and training provided by the Skilled Nurse, as appropriate, regarding the care and services identified in the plan of care.

Discharge Plans

Discharge patient to self-care.
Discharge when medical condition is stable and patient is no longer in need of skilled services.
Able to understand medication regime and care related to diagnoses.
Able to return to an independent level of care.
When goals are met.
Discharge when patient is independent in management of medical needs.
When patient knows when to notify physician.

- [ ] Written notice of discharge provided to patient

Discharge scheduled for:
mm/dd/yyyy

Discharge Plan Discussed with:

- [x] Patient/Family
- [x] Care Manager
- [x] Physician
- [ ] Other (specify):

Discharge Plan:

- [x] To self-care when goals met
- [x] To self care when max potential achieved
- [ ] To outpatient therapy with MD approval
- [ ] Others (Specify):

Comments:

PATIENT SUMMARY & CLINICAL NARRATIVE

Summary

Diego Flores-Martinez is a 38-year-old male patient referred for home health care on March 12, 2026. He has been admitted under the care of attending physician Dr. David Silverberg, with the start of care date set for March 26, 2026. The patient was discharged from a long-term care hospital within the past 14 days, which necessitated the referral for home health services. The discipline completing the assessment is registered nursing, specifically conducted by RN Kelvyn Garcia Alfonso.

Diego is considered homebound due to several factors. He experiences significant limitations in mobility and endurance post-surgery, characterized by severe pain and weakness which makes it medically contraindicated for him to leave his home. The criteria for homebound status include an inability to tolerate standing or walking for extended periods, as well as exhaustion when attempting to leave home, all of which contribute to his homebound classification.

The primary reason for the need for home health services is Diego's medical condition, specifically a displaced fracture of the head of the right radius (S52.121D), which is currently healing. This condition has resulted in functional limitations and necessitated skilled nursing care at home to assist with his recovery. In addition to his primary diagnosis, he also has a secondary diagnosis of a displaced fracture of the neck of the right radius (S52.131D) and nicotine dependence (F17.210), although he currently has no other comorbidities or co-existing conditions.

A comprehensive head-to-toe assessment reveals that Diego is experiencing moderate functional limitations. Cardiopulmonary assessment shows stable vital signs, with a temperature of 98.4°F, pulse at 72 beats per minute, blood pressure of 110/72 mmHg, and a respiratory rate of 12 breaths per minute. Gastrointestinal assessment indicates no issues with bowel or urinary incontinence. The integumentary system assessment shows a surgical wound that is currently not observable due to a non-removable dressing; however, there are no indications of unhealed pressure ulcers or stasis ulcers.

Diego's psychosocial status is relatively stable. He is oriented to time and place, as confirmed by a Brief Interview for Mental Status (BIMS) score of 15. He demonstrates no signs of cognitive decline, disorganized thinking, or altered levels of consciousness. The patient reports episodes of depression but indicates he feels neither lonely nor isolated, with a severity score of 0 on the Patient Mood Interview (PHQ-2). His behavior during the assessment was cooperative, and he appeared well-groomed.

Vital signs indicate that Diego's overall health is stable, and he exhibits no signs of dyspnea. His cognitive function is intact; he is able to comprehend and follow instructions effectively without confusion. He has a fair prognosis, but his functional ability is assessed as needing some assistance with activities of daily living (ADLs). He requires setup or cleanup help for grooming, dressing, bathing, and toileting.

Regarding mobility, Diego can walk only with supervision and requires assistance for transfers. He can ambulate with minimal assistance but is limited in his ability to perform self-care tasks independently. His functional ability is categorized as needing help in self-care, indoor ambulation, and accessing stairs. A comprehensive review of his ADLs shows that he can manage some tasks independently if provided with the necessary setup.

Socially, Diego lives with another person who serves as a non-agency caregiver. This individual is currently in need of training and supportive services to adequately assist Diego in his recovery process. Diego's social needs include transportation for medical appointments and assistance with non-medical errands, which he has struggled to meet due to his homebound status.

A review of medications indicates that Diego is not currently prescribed any oral or injectable medications, and there are no high-risk drug classes involved. This minimizes potential adverse drug interactions and side effects, contributing positively to his recovery.

Diego's primary caregiver, who lives with him, may require additional support and training to assist effectively. This highlights the need for education on emergency preparedness and care management for Diego's specific needs.

In conclusion, Diego Flores-Martinez requires skilled home health services due to his recent surgical recovery, mobility limitations, and need for assistance with daily activities. The medical necessity for skilled services is evident, and a recommended visit frequency of three times per week is appropriate to ensure adequate monitoring, education, and support through his recovery process.

CARE PLAN (PROBLEMS / GOALS / INTERVENTIONS)

Patient Care Plan / Goals / Interventions:

Problem: Vital Sign Assessment
Goals: Target Completion: 5 Week(s) Term: Long-term Status: Ongoing
Patient's vital signs will be within normal limits.
Interventions: Assigned Staff: SN
Check O2 saturation every visit for any shortness of breath on exertion, fatigue/weakness. Check VS every visit and report to PMD if patient vitals are not within the parameters.
Progress Toward Goals:
No Progress Yet

Problem: Cardiopulmonary
Goals: Target Completion: 5 Week(s) Term: Long-term Status: Ongoing
Demonstrate pursed-lip breathing exercises, deep breathing exercises, and energy-conserving measures.
Interventions: Assigned Staff: SN
Review patient/caregiver on methods to conserve energy, frequent rest periods, sitting instead of standing to perform tasks, and performing slower, purposeful movements which will limit fatigue. Instruct patient/caregiver on pursed lip breathing exercises, deep breathing exercises to alleviate dyspnea.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 5 Week(s) Term: Long-term Status: Ongoing
Patient will be free from signs/symptoms of cardiac and respiratory distress during the certification period. Verbalize understanding of symptoms of cardiac complications and when to call 911.
Interventions: Assigned Staff: SN
Reinforce patient/caregiver on sodium limitations, managing stress, avoiding smoking/alcohol intake, and compliance with prescribed medications to control blood pressure. Encourage the patient/caregiver to provide a quiet environment, limiting stimuli to promote rest. Advise the patient to get up slowly to offset the feeling of dizziness. Instruct patient/caregiver to notify SN/MD for signs/symptoms of cardiac and respiratory distress like chest pain, cyanosis, dependent edema, jugular vein distention, palpitations, wheezing, increased respiratory rate, anxiety, headache, lightheadedness, and shortness of breath.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 5 Week(s) Term: Long-term Status: Ongoing
Patient will be free from cardiopulmonary distress during the care episode. Patient's pulse and blood pressure will be within established parameters.
Interventions: Assigned Staff: SN
Reinforce patient/caregiver to check and record blood pressure, including heart rate. Assess cardiopulmonary status every visit.
Progress Toward Goals:
No Progress Yet

Problem: Displaced fracture of head of radius, subsequent for closed fracture with routine healing
Goals: Target Completion: 5 Week(s) Term: Long-term Status: Ongoing
Promote healing of the fracture and restore mobility in the affected arm.
Interventions: Assigned Staff: SN
Provide education to the patient and family on proper techniques for arm movement to prevent stiffness.
Progress Toward Goals:
No Progress Yet
Monitor pain levels and adjust activity levels accordingly to ensure comfort during rehabilitation.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 5 Week(s) Term: Long-term Status: Ongoing
Ensure the patient achieves optimal functional independence.
Interventions: Assigned Staff: SN
Schedule regular follow-ups to reassess the patient's progress and modify the care plan as necessary.
Progress Toward Goals:
No Progress Yet

Problem: Displaced fracture of the neck of the radius, subsequent for closed fracture with routine healing.
Goals: Target Completion: 5 Week(s) Term: Long-term Status: Ongoing
Achieve full range of motion in the affected wrist within 6 weeks.
Interventions: Assigned Staff: SN
Monitor pain levels and adjust therapy intensity accordingly.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 5 Week(s) Term: Long-term Status: Ongoing
Ensure pain is managed effectively to promote participation in rehabilitation.
Interventions: Assigned Staff: SN
Conduct regular assessments of pain levels and medication effectiveness.
Progress Toward Goals:
No Progress Yet
Educate the family on non-pharmacological pain management techniques, such as ice application.
Progress Toward Goals:
No Progress Yet
Collaborate with the physician for medication adjustments as necessary.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 5 Week(s) Term: Long-term Status: Ongoing
Promote independence in daily activities while ensuring safety.
Interventions: Assigned Staff: SN
Monitor the patient's progress and adjust the care plan as necessary.
Progress Toward Goals:
No Progress Yet

Problem: Nicotine dependence, cigarettes, uncomplicated
Goals: Target Completion: 5 Week(s) Term: Long-term Status: Ongoing
Reduce nicotine dependence by 50% within 6 months.
Interventions: Assigned Staff: SN
Provide education on the effects of nicotine and smoking cessation strategies to the patient and family.
Progress Toward Goals:
No Progress Yet
Implement a nicotine replacement therapy program as prescribed.
Progress Toward Goals:

No Progress Yet
Monitor and document cravings and withdrawal symptoms during home visits.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 5 Week(s) Term: Long-term Status: Ongoing
Achieve complete smoking cessation within 1 year.
Interventions: Assigned Staff: SN
Develop a personalized quit plan with the patient and family members.
Progress Toward Goals:
No Progress Yet
Provide regular follow-up visits to assess progress and adapt strategies as needed.
Progress Toward Goals:
No Progress Yet

Problem: Fall Risk
Goals: Target Completion: 5 Week(s) Term: Long-term Status: Ongoing
Skilled Nurse assessed patient's safety in environment, mobility and functional limitations to reduce falls or accidents every visit. Assessed the patient's ability to perform ADLs effectively and safely every visit. Instructed Patient/Caregiver on the importance of assessing conditions/risks that may contribute to occurrence of falls such as fatigue/ muscle weakness. Encouraged to perform exercises if tolerated for strengthening and maintenance of posture and flexibility. Home exercise program (HEP) was established, and patient/caregiver were educated on passive and active assistive range-of-motion (ROM) exercises. Skilled Nurse guided patient through supervised balance and gait training exercises to improve stability and coordination, as well as facilitating functional mobility tasks such as sit-to-stand transfers and safe ambulation techniques.
Interventions: Assigned Staff: SN
New Intervention
Progress Toward Goals:
No Progress Yet

Problem: Pressure Ulcer Prevention
Goals: Target Completion: 5 Week(s) Term: Long-term Status: Ongoing
Verbalize understanding of the importance of pressure-relieving measures.
Interventions: Assigned Staff: SN
Instruct patient/caregiver on pressure-relieving measures such as repositioning every 2 hours, float heels, pad bony prominences and methods to reduce friction and shear.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 5 Week(s) Term: Long-term Status: Ongoing
The patient's skin remains intact.
Interventions: Assigned Staff: SN
Instruct patient/caregiver to inspect skin daily and notify SN/MD for redness or open sores. Assess skin for breakdown due to risk of pressure ulcer development.
Progress Toward Goals:
No Progress Yet

Problem: Rehospitalization Risk and Medication Management
Goals: Target Completion: 5 Week(s) Term: Long-term Status: Ongoing
Patient will be free from side effects and adverse medication reactions during the entire episode. Patient/Caregiver will demonstrate medication safety and compliance. Patients will have no re-hospitalizations.
Interventions: Assigned Staff: SN
New Intervention
Progress Toward Goals:
No Progress Yet

Ongoing Care Plans within OASIS date (03/26/2026):

Assigned Staff: Garcia Alfonso, Kelvyn RN